

1 ENGROSSED SENATE AMENDMENT
TO
2 ENGROSSED HOUSE
BILL NO. 1687

By: Roe of the House

and

Rosino of the Senate

3
4
5
6
7 An Act relating to advance health care directives;
8 creating the Uniform Health Care Decisions Act of
9 2025; providing definitions; specifying capacity;
10 providing for presumption of capacity; providing for
11 overcoming *** health care professionals; providing
for responsibilities of health care professionals and
health care *** construction; providing for
application; providing for codification; and
providing an effective date.

12
13 AMENDMENT NO. 1. Page 1, strike the title, enacting clause and
14 entire bill and insert

15 "An Act relating to advance health care directives;
16 creating the Uniform Health Care Decisions Act of
17 2026; providing short title; defining terms;
18 establishing requirements to show capacity; providing
19 for presumption of capacity under certain
20 circumstances; providing for rebuttal of certain
21 presumption; requiring certain notice; establishing
22 procedures to object to certain finding; authorizing
23 judicial review of certain finding; authorizing
24 creation of health care instruction; establishing
requirements for health care instruction;
establishing requirements for creation of power of
attorney for health care; authorizing creation of
advance mental health care directive; establishing
requirements for advance mental health care
directive; establishing requirements for conflicting
health care directives; providing optional form for
advance health care directive; authorizing default
surrogate who is a member of certain classes;

1 establishing requirements for default surrogate;
2 establishing procedures for disagreement of default
3 surrogates; providing for disqualification of certain
4 persons for acting as default surrogate; authorizing
5 revocation of certain appointment, designation, or
6 instruction; establishing requirements for validity
7 of directives created out of state; directing
8 resolution of conflicts of law; establishing duties
9 of agent or default surrogate; providing for
10 commencement and ceasing of powers of agent or
11 default surrogate; prohibiting certain actions by
12 agent or default surrogate; authorizing appointment
13 of coagents and alternate agents; prohibiting
14 authorization of certain persons to serve as agent or
15 default surrogate; establishing certain duties for
16 health care professionals and health care
17 institutions; defining term; authorizing certain
18 decisions by guardians; providing immunity from civil
19 or criminal liability for certain health care
20 professionals or institutions; prohibiting certain
21 conduct; providing for award of damages; authorizing
22 issuance of certain court orders upon petition;
23 construing provisions; requiring court to consider
24 uniformity of law under certain circumstances;
providing for validity of certain advance directives;
clarifying applicability of act; requiring issuance
of court order upon certain motion; repealing 63 O.S.
2021, Sections 3101.1, 3101.2, 3101.3, 3101.4,
3101.5, 3101.6, 3101.7, 3101.8, 3101.9, 3101.10,
3101.11, as amended by Section 737, Chapter 486,
O.S.L. 2025, 3101.12, 3101.13, 3101.14, 3101.15,
3101.16, 3102.4, as amended by Section 15, Chapter
136, O.S.L. 2022, and 3102.5 (63 O.S. Supp. 2025,
Sections 3101.11 and 3102.4), which relate to the
Oklahoma Advance Directive Act; repealing Sections 1,
2, 3, as amended by Section 1, Chapter 154, O.S.L.
2023, 4, 5, as amended by Section 2, Chapter 154,
O.S.L. 2023, 6, 7, 8, 9, 10, 11, 12, and 13, Chapter
136, O.S.L. 2022 (63 O.S. Supp. 2025, Sections
3111.1, 3111.2, 3111.3, 3111.4, 3111.5, 3111.6,
3111.7, 3111.8, 3111.9, 3111.10, 3111.11, 3111.12,
and 3111.13), which relate to the Oklahoma Health
Care Agent Act; providing for codification; and
providing an effective date.

1 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

2 SECTION 1. NEW LAW A new section of law to be codified
3 in the Oklahoma Statutes as Section 3100.1 of Title 63, unless there
4 is created a duplication in numbering, reads as follows:

5 This act shall be known and may be cited as the "Uniform Health
6 Care Decisions Act of 2026".

7 SECTION 2. NEW LAW A new section of law to be codified
8 in the Oklahoma Statutes as Section 3100.2 of Title 63, unless there
9 is created a duplication in numbering, reads as follows:

10 As used in this act:

11 1. "Advance health care directive" means a power of attorney
12 for health care, health care instruction, or both. The term
13 includes an advance mental health care directive;

14 2. "Advance mental health care directive" means a power of
15 attorney for health care, health care instruction, or both, created
16 under Section 9 of this act;

17 3. "Agent" means an individual appointed under a power of
18 attorney for health care to make a health care decision for the
19 individual who made the appointment. The term includes a coagent or
20 alternate agent appointed under Section 20 of this act;

21 4. "Capacity" means having capacity under Section 3 of this
22 act;

23 5. "Cohabitant" means each of two individuals who have been
24 living together as a couple for at least one (1) year after each

1 became an adult or was emancipated and who are not married to one
2 another;

3 6. "Default surrogate" means an individual authorized under
4 Section 12 of this act to make a health care decision for another
5 individual;

6 7. "Electronic" means relating to technology having electrical,
7 digital, magnetic, wireless, optical, electromagnetic, or similar
8 capabilities;

9 8. "Family member" means a spouse, domestic partner, adult
10 child, parent, or grandparent, or an adult descendant of a spouse,
11 child, parent, or grandparent;

12 9. "Guardian" means a person appointed under law by a court to
13 make decisions regarding the personal affairs of an individual,
14 which may include health care decisions. The term does not include
15 a guardian ad litem;

16 10. "Health care" means care or treatment or a service or
17 procedure to maintain, monitor, diagnose, or otherwise affect an
18 individual's physical or mental illness, injury, or condition. The
19 term includes mental health care;

20 11. "Health care decision" means a decision made by an
21 individual or the individual's surrogate regarding the individual's
22 health care, including:

23 a. selection or discharge of a health care professional
24 or health care institution,

- b. approval or disapproval of a diagnostic test, surgical procedure, medication, therapeutic intervention, or other health care,
- c. direction to provide, withhold, or withdraw mechanical ventilation or other health care, and
- d. direction to provide, withhold, or withdraw artificially administered hydration or nutrition, subject to the limitations of the Hydration and Nutrition for Incompetent Patients Act;

12. "Health care institution" means a facility or agency licensed, certified, or otherwise authorized or permitted by other law to provide health care in this state in the ordinary course of business;

13. "Health care instruction" means a direction, whether or not in a record, made by an individual that indicates the individual's goals, preferences, or wishes concerning the provision, withholding, or withdrawal of health care. The term includes a direction intended to be effective if a specified condition arises;

14. "Health care professional" means a physician or other individual licensed, certified, or otherwise authorized or permitted by other law of this state to provide health care in this state in the ordinary course of business or the practice of the physician's or individual's profession;

15. "Individual" means an adult or emancipated minor;

1 16. "Life-sustaining care" and "life-sustaining treatment" mean
2 health care that, in reasonable medical judgment, will be most
3 likely to be effective in ameliorating an individual's life-
4 threatening conditions;

5 17. "Mental health care" means care or treatment or a service
6 or procedure to maintain, monitor, diagnose, or otherwise affect an
7 individual's mental illness or other psychiatric, psychological, or
8 psychosocial condition;

9 18. "Nursing home" means a nursing facility as defined in
10 Section 1919(a)(1) of the Social Security Act, 42 U.S.C., Section
11 1396r(a)(1), as amended, or skilled nursing facility as defined in
12 Section 1819(a)(1) of the Social Security Act, 42 U.S.C., Section
13 1395i-3(a)(1), as amended;

14 19. "Person" means an individual, estate, business or nonprofit
15 entity, government or governmental subdivision, agency, or
16 instrumentality, or other legal entity;

17 20. "Person interested in the welfare of the individual" means:

- 18 a. the individual's surrogate,
- 19 b. a family member of the individual,
- 20 c. the cohabitant of the individual,
- 21 d. a public entity providing health care case management
- 22 or protective services to the individual,
- 23
- 24

1 e. a person appointed under other law to make decisions
2 for the individual under a power of attorney for
3 finances, or

4 f. a person that has an ongoing personal or professional
5 relationship with the individual, including a person
6 that has provided educational or health care services
7 or supported decision making to the individual;

8 21. "Physician" means an individual authorized to practice
9 medicine from the State Board of Medical Licensure and Supervision
10 or the State Board of Osteopathic Examiners;

11 22. "Power of attorney for health care" means a record in which
12 an individual appoints an agent to make health care decisions for
13 the individual;

14 23. "Reasonable medical judgment" means a health care judgment
15 that would be made by a reasonably prudent physician knowledgeable
16 about the medical conditions involved and the treatment
17 possibilities with respect to such conditions;

18 24. "Reasonably available" means being able to be contacted
19 without undue effort and being willing and able to act in a timely
20 manner considering the urgency of an individual's health care
21 situation. When used to refer to an agent or default surrogate, the
22 term includes being willing and able to comply with the duties under
23 Section 17 of this act in a timely manner considering the urgency of
24 an individual's health care situation;

1 25. "Record" means information:

- 2 a. inscribed on a tangible medium, or
- 3 b. stored in an electronic or other medium and
- 4 retrievable in perceivable form;

5 26. "Responsible health care professional" means:

- 6 a. a health care professional designated by an individual
- 7 or the individual's surrogate to have primary
- 8 responsibility for the individual's health care or for
- 9 overseeing a course of treatment, or
- 10 b. in the absence of a designation under subparagraph a
- 11 of this paragraph, or if the professional designated
- 12 under subparagraph a of this paragraph is not
- 13 reasonably available, a health care professional who
- 14 has primary responsibility for overseeing the
- 15 individual's health care or for overseeing a course of
- 16 treatment;

17 27. "Sign" means with present intent to authenticate or adopt a
18 record:

- 19 a. executing or adopting a tangible symbol, or
- 20 b. attaching to or logically associating with the record
- 21 an electronic symbol, sound, or process;

22 28. "State" means a state of the United States, the District of
23 Columbia, the Commonwealth of Puerto Rico, the United States Virgin
24 Islands, or any other territory, or possession subject to the

1 jurisdiction of the United States. The term includes a federally
2 recognized Indian tribe;

3 29. "Supported decision making" means assistance, from one or
4 more persons of an individual's choosing, that helps the individual
5 make or communicate a decision, including by helping the individual
6 understand the nature and consequences of the decision; and

7 30. "Surrogate" means:

8 a. an agent,

9 b. a default surrogate, or

10 c. a guardian authorized to make health care decisions.

11 SECTION 3. NEW LAW A new section of law to be codified
12 in the Oklahoma Statutes as Section 3100.3 of Title 63, unless there
13 is created a duplication in numbering, reads as follows:

14 A. An individual has capacity for the purpose of this act if
15 the individual:

16 1. Is willing and able to communicate a decision independently
17 or with appropriate services, technological assistance, supported
18 decision making, or other reasonable accommodation; and

19 2. In making or revoking:

20 a. a health care decision, understands the nature and
21 consequences of the decision, including the primary
22 risks and benefits of the decision,

23 b. a health care instruction, understands the nature and
24 consequences of the instruction, including the primary

1 risks and benefits of the choices expressed in the
2 instruction, and

3 c. an appointment of an agent under a health care power
4 of attorney or identification of a default surrogate
5 under paragraph 1 of subsection B of Section 12 of
6 this act, recognizes the identity of the individual
7 being appointed or identified and understands the
8 general nature of the relationship of the individual
9 making the appointment or identification with the
10 individual being appointed or identified.

11 B. The right of an individual who has capacity to make a
12 decision about the individual's health care is not affected by
13 whether the individual creates or revokes an advance health care
14 directive.

15 SECTION 4. NEW LAW A new section of law to be codified
16 in the Oklahoma Statutes as Section 3100.4 of Title 63, unless there
17 is created a duplication in numbering, reads as follows:

18 A. An individual is presumed to have capacity to make or revoke
19 a health care decision, health care instruction, and power of
20 attorney for health care unless:

- 21 1. A court has found the individual lacks capacity to do so; or
- 22 2. The presumption is rebutted under subsection B of this
- 23 section.

1 B. Subject to Sections 5 and 6 of this act, a presumption under
2 subsection A of this section may be rebutted by a finding that the
3 individual lacks capacity:

4 1. Subject to subsection C of this section, made on the basis
5 of a contemporaneous examination by any of the following:

- 6 a. a physician,
- 7 b. a psychologist licensed or otherwise authorized to
8 practice in this state,
- 9 c. an individual with training and expertise in the
10 finding of lack of capacity who is licensed or
11 otherwise authorized to practice in this state as an
12 Advanced Practice Registered Nurse, a physician
13 assistant, or a social worker, or
- 14 d. a responsible health care professional not described
15 in subparagraph a, b, or c of this paragraph if:
 - 16 (1) the individual about whom the finding is to be
17 made is experiencing a health condition requiring
18 a decision regarding health care treatment to be
19 made promptly to avoid loss of life or serious
20 harm to the health of the individual, and
 - 21 (2) an individual listed in subparagraph a, b, or c
22 of this paragraph is not reasonably available;

1 2. Made in accordance with accepted standards of the profession
2 and the scope of practice of the individual making the finding and
3 to a reasonable degree of certainty; and

4 3. Documented in a record signed by the individual making the
5 finding that includes an opinion of the cause, nature, extent, and
6 probable duration of the lack of capacity.

7 C. The finding under subsection B of this section may not be
8 made by:

9 1. A family member of the individual presumed to have capacity;

10 2. The cohabitant of the individual or a descendant of the
11 cohabitant; or

12 3. The individual's surrogate, a family member of the
13 surrogate, or a descendant of the surrogate.

14 D. If the finding under subsection B of this section was based
15 on a condition the individual no longer has or a responsible health
16 care professional subsequently has good cause to believe the
17 individual has capacity, the individual is presumed to have capacity
18 unless a court finds the individual lacks capacity or the
19 presumption is rebutted under subsection B of this section.

20 SECTION 5. NEW LAW A new section of law to be codified
21 in the Oklahoma Statutes as Section 3100.5 of Title 63, unless there
22 is created a duplication in numbering, reads as follows:

23 A. As soon as reasonably feasible, an individual who makes a
24 finding under subsection B of Section 4 of this act shall inform the

1 individual about whom the finding was made or the individual's
2 responsible health care professional of the finding.

3 B. As soon as reasonably feasible, a responsible health care
4 professional who is informed of a finding under subsection B of
5 Section 4 of this act shall inform the individual about whom the
6 finding was made and the individual's surrogate.

7 C. An individual found under subsection B of Section 4 of this
8 act to lack capacity may object to the finding:

9 1. By orally informing a responsible health care professional;

10 2. In a record provided to a responsible health care
11 professional or the health care institution in which the individual
12 resides or is receiving care; or

13 3. By another act that clearly indicates the individual's
14 objection.

15 D. If the individual objects under subsection C of this
16 section, the finding under subsection B of Section 4 of this act is
17 not sufficient to rebut a presumption of capacity under subsection A
18 of Section 4 of this act and the individual shall be treated as
19 having capacity unless:

20 1. The individual withdraws the objection;

21 2. A court finds the individual lacks the presumed capacity;

22 3. The individual is experiencing a health condition requiring
23 a decision regarding health care treatment to be made promptly to
24

1 avoid imminent loss of life or serious harm to the health of the
2 individual; or

3 4. Subject to subsection B of this section, the finding is
4 confirmed by a second finding made by an individual authorized under
5 paragraph 1 of subsection B of Section 4 of this act who:

6 a. did not make the first finding,

7 b. is not a family member of the individual who made the
8 first finding, and

9 c. is not the cohabitant of the individual who made the
10 first finding or a descendant of the cohabitant.

11 E. Neither a finding in the circumstances described in
12 paragraph 3 of subsection D of this section nor a second finding
13 that the individual lacks capacity under paragraph 4 of subsection D
14 of this section shall be sufficient to rebut the presumption of
15 capacity if the individual is requesting the provision or
16 continuation of life-sustaining treatment and the finding is being
17 used to make a decision to withhold or withdraw the treatment.

18 F. As soon as reasonably feasible, a health care professional
19 who is informed of an objection under subsection C of this section
20 shall:

21 1. Communicate the objection to a responsible health care
22 professional; and

23 2. Document the objection and the date of the objection in the
24 individual's medical record or communicate the objection and the

1 date of the objection to an administrator with responsibility for
2 medical records of the health care institution providing health care
3 to the individual, who shall document the objection and the date of
4 the objection in the individual's medical record.

5 SECTION 6. NEW LAW A new section of law to be codified
6 in the Oklahoma Statutes as Section 3100.6 of Title 63, unless there
7 is created a duplication in numbering, reads as follows:

8 A. An individual found under subsection B of Section 4 of this
9 act to lack capacity, a responsible health care professional, the
10 health care institution providing health care to the individual, or
11 a person interested in the welfare of the individual may petition
12 the court in the respective county in which the individual resides
13 or is located to determine whether the individual lacks capacity.

14 B. The court in which a petition under subsection A of this
15 section is filed shall appoint legal counsel to represent the
16 individual if the individual does not have legal counsel. The court
17 shall hear the petition as soon as possible, but not later than
18 seven (7) days after the petition is filed. As soon as possible,
19 but not later than seven (7) days after the hearing, the court shall
20 determine whether the individual lacks capacity. The court may
21 determine that the individual lacks capacity only if the court finds
22 by clear and convincing evidence that the individual lacks capacity.

1 SECTION 7. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.7 of Title 63, unless there
3 is created a duplication in numbering, reads as follows:

4 A. An individual may create a health care instruction that
5 expresses the individual's preferences for future health care,
6 including preferences regarding:

7 1. Health care professionals or health care institutions;

8 2. How a health care decision will be made and communicated;

9 3. Persons that should or should not be consulted regarding a
10 health care decision;

11 4. A person to serve as guardian for the individual if one is
12 appointed; and

13 5. An individual to serve as a default surrogate.

14 B. A health care professional to whom an individual
15 communicates or provides an instruction under subsection A of this
16 section shall document the instruction and the date of the
17 instruction in the individual's medical record or communicate the
18 instruction and date of the instruction to an administrator with
19 responsibility for medical records of the health care institution
20 providing health care to the individual, who shall document the
21 instruction and the date of the instruction in the individual's
22 medical record.

23 C. A health care instruction made by an individual that
24 conflicts with an earlier health care instruction made by the

1 individual, including an instruction documented in a medical order,
2 revokes the earlier instruction to the extent of the conflict.

3 D. A health care instruction may be in the same record as a
4 power of attorney for health care.

5 SECTION 8. NEW LAW A new section of law to be codified
6 in the Oklahoma Statutes as Section 3100.8 of Title 63, unless there
7 is created a duplication in numbering, reads as follows:

8 A. An individual may create a power of attorney for health care
9 to appoint an agent to make health care decisions for the
10 individual.

11 B. An individual shall be disqualified from acting as agent for
12 an individual who lacks capacity to make health care decisions if:

13 1. A court finds that the potential agent poses a danger to the
14 individual's well-being, even if the court does not issue a
15 restraining order against the potential agent; or

16 2. The potential agent is an owner, operator, employee, or
17 contractor of a nursing home, or other residential care facility, in
18 which the individual resides or is receiving care, unless the owner,
19 operator, employee, or contractor is a family member of the
20 individual, the cohabitant of the individual, or a descendant of the
21 cohabitant.

22 C. A health care decision made by an agent is effective without
23 judicial approval.

1 D. A power of attorney for health care shall be in a record,
2 signed by the individual creating the power, and signed by an adult
3 witness who:

4 1. Reasonably believes the act of the individual to create the
5 power of attorney is voluntary and knowing; and

6 2. Is not:

7 a. the agent appointed by the individual,

8 b. the agent's spouse or cohabitant, or

9 c. if the individual resides or is receiving care in a
10 nursing home, or other residential care facility, the
11 owner, operator, employee, or contractor of the
12 nursing home or other residential care facility; and

13 3. Is present when the individual signs the power of attorney
14 or when the individual represents that the power of attorney
15 reflects the individual's wishes.

16 E. A witness under subsection D of this section is considered
17 present if the witness and the individual are:

18 1. Physically present in the same location;

19 2. Using electronic means that allow for real time audio and
20 visual transmission and communication in real time to the same
21 extent as if the witness and the individual were physically present
22 in the same location; or

23 3. Able to speak to and hear each other in real time through
24 audio connection if:

- a. the identity of the individual is personally known to the witness, or
- b. the witness is able to authenticate the identity of the individual by receiving accurate answers from the individual that enables the authentication.

F. A power of attorney for health care may include a health care instruction.

SECTION 9. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.9 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. An individual may create an advance health care directive that addresses only mental health care for the individual. The advance mental health care directive may include a health care instruction, a power of attorney for health care, or both.

B. A health care instruction under this section may include the individual's:

1. General philosophy and objectives regarding mental health care; and

2. Specific goals, preferences, and wishes regarding the provision, withholding, or withdrawal of a form of mental health care, including:

- a. preferences regarding professionals, programs, and facilities,

- b. admission to a mental health facility, including duration of admission,
- c. preferences regarding medications,
- d. refusal to accept a specific type of mental health care, including a medication, and
- e. preferences regarding crisis intervention.

C. A power of attorney for health care under this section may appoint an agent to make decisions only for mental health care.

D. An individual may direct in an advance mental health care directive that, if the individual is experiencing a psychiatric or psychological event specified in the directive, the individual may not revoke the directive or a part of the directive.

E. If an advance mental health care directive includes a direction under subsection D of this section, the advance mental health care directive shall be in a record that is separate from any other advance health care directive created by the individual and signed by the individual creating the advance mental health care directive and at least two adult witnesses who:

1. Attest that to the best of their knowledge the individual:

- a. understood the nature and consequences of the direction, including its risks and benefits, and
- b. made the direction voluntarily and without coercion or undue influence;

2. Are not:

- a. the agent appointed by the individual,
- b. the agent's spouse, or cohabitant, and
- c. if the individual resides in a nursing home, or other residential care facility, the owner, operator, employee, or contractor of the nursing home or other residential care facility; and

3. Are physically present in the same location as the individual.

SECTION 10. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 3100.10 of Title 63, unless there is created a duplication in numbering, reads as follows:

A. If a direction in an advance mental health care directive of an individual conflicts with a direction in another advance health care directive of the individual, the later direction revokes the earlier direction to the extent of the conflict.

B. An appointment of an agent to make decisions only for mental health care for an individual does not revoke an earlier appointment of an agent to make other health care decisions for the individual. A later appointment revokes the authority of an agent under the earlier appointment to make decisions about mental health care unless otherwise specified in the power of attorney making the later appointment.

C. An appointment of an agent to make health care decisions for an individual other than decisions about mental health care made

1 after appointment of an agent authorized to make only mental health
2 care decisions does not revoke the appointment of the agent
3 authorized to make only mental health care decisions.

4 SECTION 11. NEW LAW A new section of law to be codified
5 in the Oklahoma Statutes as Section 3100.11 of Title 63, unless
6 there is created a duplication in numbering, reads as follows:

7 The following form may be used to create an advance health care
8 directive:

9 ADVANCE HEALTH CARE DIRECTIVE

10 HOW YOU CAN USE THIS FORM

11 You can use this form if you wish to name someone to make health
12 care decisions for you in case you cannot make decisions for
13 yourself. This is called giving the person a power of attorney for
14 health care. This person is called your agent.

15 You can also use this form to state your wishes, preferences,
16 and goals for health care, and to say if you want to be an organ
17 donor after you die.

18 YOUR NAME AND DATE OF BIRTH

19 Name:

20 Date of birth:

21 PART A: NAMING AN AGENT

22 This part lets you name someone else to make health care
23 decisions for you. You may leave any item blank.

24 1. NAMING AN AGENT

1 I want the following person to make health care decisions for me
2 if I cannot make decisions for myself:

3 Name:

4 Optional contact information (it is helpful to include
5 information such as address, phone, and email):

6 2. NAMING AN ALTERNATE AGENT

7 I want the following person to make health care decisions for me
8 if I cannot and my agent is not able or available to make them for
9 me:

10 Name:

11 Optional contact information (it is helpful to include
12 information such as address, phone, and email):

13 3. LIMITING YOUR AGENT'S AUTHORITY

14 I give my agent the power to make all health care decisions for
15 me if I cannot make those decisions for myself, except the
16 following:

17 (If you do not add a limitation here, your agent will be able to
18 make all health care decisions that an agent is permitted to make
19 under state law. State law does not permit your agent to withhold
20 or withdraw food and liquids needed for you to stay alive except as
21 you specify in this document or by any other means the law
22 provides.)

23 PART B: HEALTH CARE INSTRUCTIONS

24

1 This part lets you state your priorities for health care and to
2 state types of health care you do and do not want.

3 1. INSTRUCTIONS ABOUT LIFE-SUSTAINING TREATMENT

4 This section gives you the opportunity to say how you want your
5 agent to act while making decisions for you. You may mark or
6 initial each choice. You also may leave any choice blank.

7 Treatment. Medical treatment needed to keep me alive but not
8 needed for comfort or any other purpose should (mark or initial all
9 that apply):

10 (____) Always be given to me. (If you mark or initial this
11 choice, you should not mark or initial other choices in this
12 "treatment" section.)

13 (____) Not be given to me if I have a condition that is not
14 curable and is expected to cause me to die within two weeks, even if
15 treated.

16 (____) Not be given to me if I am unconscious and I am not
17 expected to be conscious again.

18 (____) Not be given to me if I have a medical condition from
19 which I am not expected to recover that prevents me from
20 communicating with people I care about, caring for myself, and
21 recognizing family and friends.

22 (____) Other (write what you want or do not want):

23 Food and liquids. If I can't swallow and staying alive requires
24 me to get food or liquids through a tube or other means for the rest

1 of my life, then food or liquids should (mark or initial all that
2 apply):

3 (____) Always be given to me. (If you mark or initial this
4 choice, you should not mark or initial other choices in this "food
5 and liquids" section.)

6 (____) Not be given to me if I have a condition that is not
7 curable and is expected to cause me to die within two weeks, even if
8 treated.

9 (____) Not be given to me if I am unconscious and am not
10 expected to be conscious again.

11 (____) Not be given to me if I have a medical condition from
12 which I am not expected to recover that prevents me from
13 communicating with people I care about, caring for myself, and
14 recognizing family and friends.

15 (____) Other (write what you want or do not want):

16 Pain relief. If I am in significant pain, care that will keep
17 me comfortable but is likely to shorten my life should (mark or
18 initial all that apply):

19 (____) Always be given to me. (If you mark or initial this
20 choice, you should not mark or initial other choices in this "pain
21 relief" section.)

22 (____) Never be given to me. (If you mark or initial this
23 choice, you should not mark or initial other choices in this "pain
24 relief" section.)

1 (____) Be given to me if I have a condition that is not curable
2 and is expected to cause me to die soon, even if treated.

3 (____) Be given to me if I am unconscious and am not expected
4 to be conscious again.

5 (____) Be given to me if I have a medical condition from which
6 I am not expected to recover that prevents me from communicating
7 with people I care about, caring for myself, and recognizing family
8 and friends.

9 (____) Other (write what you want or do not want):

10 2. MY PRIORITIES

11 You can use this section to indicate what is important to you,
12 and what is not important to you. This information can help your
13 agent make decisions for you if you cannot. It also helps others
14 understand your preferences.

15 You may leave any choice blank.

16 My priorities:

17 _____
18 _____
19 _____
20 _____

21 Having my agent talk with my family before making decisions
22 about my care is:

23 (____) Very important

24 (____) Somewhat important

(____) Not important

Having my agent talk with my friends before making decisions about my care is:

(____) Very important

(____) Somewhat important

(____) Not important

3. OTHER INSTRUCTIONS

You can write in this section more information about your goals, values, and preferences for treatment, including care you want or do not want. You can also use this section to name anyone who you do not want to make decisions for you under any conditions.

PART C: OPTIONAL SPECIAL POWERS AND GUIDANCE

This part lets you give your agent additional powers, and to provide more guidance about your wishes. You may mark or initial each choice. You also may leave any choice blank.

1. OPTIONAL SPECIAL POWERS

My agent can do the following things ONLY if I have marked or initialed them below:

(____) Consent to withholding of food or liquid provided through a tube or by other means and required to keep me alive. (If I do not mark or initial this choice, my agent MAY NOT prevent me from receiving such food or liquid.)

(____) Admit me as a voluntary patient to a facility for mental health treatment for up to ____ days (write in the number of days

1 you want like 7, 14, 30, or another number). (If I do not mark or
2 initial this choice, my agent MAY NOT admit me as a voluntary
3 patient to this type of facility.)

4 (____) Place me in a nursing home, other than a short-term
5 placement for rehabilitation following hospitalization, even if my
6 needs can be met somewhere else. (If I do not mark or initial this
7 choice, my agent MAY NOT do this.)

8 2. ACCESS TO MY HEALTH INFORMATION

9 My agent may obtain, examine, and share information about my
10 health needs and health care if I am not able to make decisions for
11 myself. If I mark or initial below, my agent may also do that at
12 any time my agent thinks it will help me.

13 (____) I give my agent permission to obtain, examine, and share
14 information about my health needs and health care whenever my agent
15 thinks it will help me.

16 3. FLEXIBILITY FOR MY AGENT

17 Mark or initial below if you want to give your agent flexibility
18 in following instructions you provide in this form. If you do not,
19 your agent must follow the instructions even if your agent thinks
20 something else would be better for you.

21 (____) I give my agent permission to be flexible in applying
22 these instructions if my agent thinks it would be in my best
23 interest based on what my agent knows about me.

24 4. NOMINATION OF GUARDIAN

1 You can say who you would want as your guardian if you needed
2 one. A guardian is a person appointed by a court to make decisions
3 for someone who cannot make decisions. Filling this out does NOT
4 mean you want or need a guardian.

5 If a court appoints a guardian to make personal decisions for
6 me, I want the court to choose:

7 (____) My agent named in this form. If my agent cannot be a
8 guardian, I want the alternate agent named in this form.

9 (____) Other (write who you would want and their contact
10 information):

11 PART D: ORGAN DONATION

12 This part lets you donate your organs after you die. You may
13 leave any item blank. If you leave both choices under "DONATION"
14 blank, your health care agent may decide whether your organs will be
15 donated.

16 1. DONATION

17 You may mark or initial only one choice.

18 (____) I donate my organs, tissues, and other body parts after
19 I die, even if it requires maintaining treatments that conflict with
20 other instructions I have put in this form, EXCEPT for those I list
21 below (list any body parts you do NOT want to donate):

22 _____
23 _____

24

1
2
3
4
5
6
7
8
9
10
11
12
13
14
15
16
17
18
19
20
21
22
23
24

 (____) I do not want my organs, tissues, or body parts donated
to anybody for any reason. (If you mark or initial this choice, you
should skip the "PURPOSE OF DONATION" section.)

2. PURPOSE OF DONATION

You may mark or initial all that apply. (If you do not mark or
initial any of the purposes below, your donation can be used for all
of them.)

- (____) Transplant
- (____) Therapy
- (____) Research
- (____) Education
- (____) All of the above

PART E: SIGNATURES

YOUR SIGNATURE

Sign your name:

Today's date:

City/Town/Village and State (optional):

SIGNATURE OF A WITNESS

You need a witness if you are using this form to name an agent.
The witness must be an adult and cannot be the person you are naming
as agent or the agent's spouse, or someone the agent lives with as a
couple. If you live or are receiving care in a nursing home, the

1 witness cannot be an employee or contractor of the home or someone
2 who owns or runs the home.

3 Name of Witness:

4 Signature of Witness:

5 (Only sign as a witness if you think the person signing above is
6 doing it voluntarily.)

7 Date witness signed:

8 PART F: INFORMATION FOR AGENTS

9 1. If this form names you as an agent, you can make decisions
10 about health care for the person who named you when the person
11 cannot make their own.

12 2. If you make a decision for the person, follow any
13 instructions the person gave, including any in this form.

14 3. If you do not know what the person would want, make the
15 decision that you think is in the person's best interest. To figure
16 out what is in the person's best interest, consider the person's
17 values, preferences, and goals if you know them or can learn them.
18 Some of these preferences may be in this form. You should also
19 consider any behavior or communication from the person that
20 indicates what the person currently wants.

21 4. If this form names you as an agent, you can also get and
22 share the person's health information. Unless the person has said
23 so in this form, you can get or share this information only when the
24 person cannot make decisions about the person's health care.

1 SECTION 12. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.12 of Title 63, unless
3 there is created a duplication in numbering, reads as follows:

4 A. A default surrogate may make a health care decision for an
5 individual who lacks capacity to make health care decisions and for
6 whom an agent, or guardian authorized to make health care decisions,
7 has not been appointed or is not reasonably available.

8 B. Unless the individual has an advance health care directive
9 that indicates otherwise, a member of the following classes, in
10 descending order of priority, who is reasonably available and not
11 disqualified under Section 14 of this act, may act as a default
12 surrogate for the individual:

13 1. An adult the individual has identified, other than in a
14 power of attorney for health care, to make a health care decision
15 for the individual if the individual cannot make the decision;

16 2. The individual's spouse, unless:

17 a. a petition for annulment, divorce, dissolution of
18 marriage, legal separation, or termination has been
19 filed and not dismissed or withdrawn,

20 b. a decree of annulment, divorce, dissolution of
21 marriage, legal separation, or termination has been
22 issued,

23 c. the individual and the spouse have agreed in a record
24 to a legal separation, or

d. the spouse has abandoned the individual for more than one (1) year;

3. The individual's adult child or parent;

4. The individual's cohabitant;

5. The individual's adult sibling;

6. The individual's adult grandchild or grandparent;

7. An adult not listed in paragraphs 1 through 6 of this subsection who has assisted the individual with supported decision making routinely during the preceding six (6) months;

8. The individual's adult stepchild not listed in paragraphs 1 through 7 of this subsection whom the individual actively parented during the stepchild's minor years and with whom the individual has an ongoing relationship; or

9. An adult not listed in paragraphs 1 through 8 of this subsection who has exhibited special care and concern for the individual and is familiar with the individual's personal values.

C. A responsible health care professional may require an individual who assumes authority to act as a default surrogate to provide a declaration in a record under penalty of perjury stating facts and circumstances reasonably sufficient to establish the authority.

D. If a responsible health care professional reasonably determines that an individual who assumed authority to act as a default surrogate is not willing or able to comply with a duty under

1 Section 17 of this act or fails to comply with the duty in a timely
2 manner, the professional may recognize the individual next in
3 priority under subsection B of this section as the default
4 surrogate.

5 E. A health care decision made by a default surrogate is
6 effective without judicial approval.

7 F. Prior to making a health care decision for an individual
8 pursuant to subsection A of this section, a responsible health care
9 professional may require a default surrogate to provide to the
10 responsible health care professional a signed copy of the following
11 statement to be entered into the patient's medical record:

12 "I hereby certify that:

13 I have not been convicted of, pleaded guilty to, or pleaded no
14 contest to the crimes of abuse, verbal abuse, neglect, or financial
15 exploitation by a caregiver; exploitation of an elderly person or
16 disabled adult; or abuse, neglect, exploitation, or sexual abuse of
17 a child;

18 I have not been found to have committed abuse, verbal abuse, or
19 exploitation by a final investigative finding of the State
20 Department of Health or the Department of Human Services or by a
21 finding of an administrative law judge, unless it was overturned on
22 appeal; and I have not been criminally charged as a person
23 responsible for the care of a vulnerable adult with a crime
24 resulting in the death or near death of a vulnerable adult."

1 SECTION 13. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.13 of Title 63, unless

3 there is created a duplication in numbering, reads as follows:

4 A. A default surrogate who assumes authority under Section 12
5 of this act shall inform a responsible health care professional if
6 two or more members of a class under subsection B of Section 12 of
7 this act have assumed authority to act as default surrogates and the
8 members do not agree on a health care decision.

9 B. A responsible health care professional shall comply with the
10 decision of a majority of the members of the class with highest
11 priority under subsection B of Section 12 of this act who have
12 communicated their views to the professional and who the
13 professional reasonably believes are acting consistently with their
14 duties under Section 17 of this act.

15 C. If a responsible health care professional is informed that
16 the members of the class who have communicated their views to the
17 professional are evenly divided concerning the health care decision,
18 the professional shall make a reasonable effort to solicit the views
19 of members of the class who are reasonably available but have not
20 yet communicated their views to the professional. The professional,
21 after the solicitation, shall comply with the decision of a majority
22 of the members who have communicated their views to the professional
23 and who the professional reasonably believes are acting consistently
24 with their duties under Section 17 of this act.

1 D. If the class remains evenly divided after the effort is made
2 under subsection C of this section, the health care decision shall
3 be made as provided by other laws of this state regarding the
4 treatment of an individual who is found to lack capacity.

5 E. Nothing in this section shall be construed to supersede or
6 limit the remedies available under Section 27 of this act.

7 SECTION 14. NEW LAW A new section of law to be codified
8 in the Oklahoma Statutes as Section 3100.14 of Title 63, unless
9 there is created a duplication in numbering, reads as follows:

10 A. An individual for whom a health care decision would be made
11 may disqualify another individual from acting as default surrogate
12 for the first individual. The disqualification shall be in a record
13 signed by the first individual or communicated verbally or
14 nonverbally to the individual being disqualified, another
15 individual, or a responsible health care professional.
16 Disqualification under this subsection shall be effective even if
17 made by an individual who lacks capacity to make an advance
18 directive if the individual clearly communicates a desire that the
19 individual being disqualified cannot make health care decisions for
20 the individual.

21 B. An individual is disqualified from acting as a default
22 surrogate for an individual who lacks capacity to make health care
23 decisions if:
24

1 1. A court finds that the potential default surrogate poses a
2 danger to the individual's well-being, even if the court does not
3 issue a restraining order against the potential surrogate;

4 2. The potential default surrogate is an owner, operator,
5 employee, or contractor of a nursing home, or other residential care
6 facility, in which the individual is residing or receiving care
7 unless the owner, operator, employee, or contractor is a family
8 member of the individual, the cohabitant of the individual, or a
9 descendant of the cohabitant; or

10 3. The potential default surrogate refuses to provide a timely
11 declaration under subsection C or F of Section 12 of this act.

12 SECTION 15. NEW LAW A new section of law to be codified
13 in the Oklahoma Statutes as Section 3100.15 of Title 63, unless
14 there is created a duplication in numbering, reads as follows:

15 A. An individual may revoke the appointment of an agent, the
16 designation of a default surrogate, or a health care instruction in
17 whole or in part, unless:

18 1. A court finds the individual lacks capacity to do so;

19 2. The individual is found under subsection B of Section 4 of
20 this act to lack capacity to do so, and if the individual objects to
21 the finding, the finding is confirmed under paragraph 4 of
22 subsection D of Section 5 of this act and is not revoked under
23 Section 6 of this act; or

1 3. The individual created an advance mental health care
2 directive that includes the provision under subsection D of Section
3 9 of this act, and the individual is experiencing the psychiatric or
4 psychological event specified in the directive.

5 B. Revocation under subsection A of this section may be by any
6 act of the individual that clearly indicates that the individual
7 intends to revoke the appointment, designation, or instruction,
8 including an oral statement to a health care professional.

9 C. Except as provided in Section 10 of this act, an advance
10 health care directive of an individual that conflicts with another
11 advance health care directive of the individual revokes the earlier
12 directive to the extent of the conflict.

13 D. Unless otherwise provided in an individual's advance health
14 care directive appointing an agent, the appointment of a spouse of
15 an individual as agent for the individual is revoked if:

16 1. A petition for annulment, divorce, dissolution of marriage,
17 legal separation, or termination has been filed and not dismissed or
18 withdrawn;

19 2. A decree of annulment, divorce, dissolution of marriage,
20 legal separation, or termination has been issued;

21 3. The individual and the spouse have agreed in a record to a
22 legal separation; or

23 4. The spouse has abandoned the individual for more than one
24 (1) year.

1 SECTION 16. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.16 of Title 63, unless
3 there is created a duplication in numbering, reads as follows:

4 A. Subject to subsection C of Section 19 of this act, an
5 advance health care directive created outside this state is valid if
6 it complies with:

7 1. The law of the state specified in the directive or, if a
8 state is not specified, the state in which the individual created
9 the directive; or

10 2. The provisions of this act.

11 B. A person may assume without inquiry that an advance health
12 care directive is genuine, valid, and still in effect, and may
13 implement and rely on it, unless the person has good cause to
14 believe the directive is invalid or has been revoked.

15 C. An advance health care directive, revocation of a directive,
16 or a signature on a directive or revocation may not be denied legal
17 effect or enforceability solely because it is in electronic form.

18 D. Evidence relating to an advance health care directive,
19 revocation of a directive, or a signature on a directive or
20 revocation may not be excluded in a proceeding solely because the
21 evidence is in electronic form.

22 E. This act shall not affect the validity of an electronic
23 record or signature that is valid under Section 15-101 et seq. of
24 Title 12A of the Oklahoma Statutes.

1 F. Except as provided in subsections D and E of Section 19,
2 Section 21, subsection H of Section 22, and subsections A and D of
3 Section 28 of this act, if this act conflicts with any other law of
4 this state relating to the creation, execution, implementation, or
5 revocation of an advance health care directive, the provisions of
6 this act shall prevail.

7 SECTION 17. NEW LAW A new section of law to be codified
8 in the Oklahoma Statutes as Section 3100.17 of Title 63, unless
9 there is created a duplication in numbering, reads as follows:

10 A. An agent or default surrogate has a fiduciary duty to the
11 individual for whom the agent or default surrogate is acting when
12 exercising or purporting to exercise a power under Section 18 of
13 this act.

14 B. An agent or default surrogate shall make a health care
15 decision in accordance with the direction of the individual in an
16 advance health care directive and other goals, preferences, and
17 wishes of the individual to the extent known or reasonably
18 ascertainable by the agent or default surrogate.

19 C. If there is not a direction in an advance health care
20 directive and the goals, preferences, and wishes of the individual
21 regarding a health care decision are not known or reasonably
22 ascertainable by the agent or default surrogate, the agent or
23 default surrogate shall make the decision in accordance with the
24

1 agent's or default surrogate's reasonable determination of the
2 individual's best interest.

3 D. In determining the individual's best interest under
4 subsection C of this section, the agent or default surrogate shall:

5 1. Give primary consideration to the individual's
6 contemporaneous communications, including verbal and nonverbal
7 expressions;

8 2. Consider the individual's values to the extent known or
9 reasonably ascertainable by the agent or default surrogate; and

10 3. Consider the risks and benefits of the potential health care
11 decision.

12 E. As soon as reasonably feasible, an agent or default
13 surrogate who is informed of a revocation of an advance health care
14 directive or disqualification of the agent or default surrogate
15 shall communicate the revocation or disqualification to a
16 responsible health care professional.

17 SECTION 18. NEW LAW A new section of law to be codified
18 in the Oklahoma Statutes as Section 3100.18 of Title 63, unless
19 there is created a duplication in numbering, reads as follows:

20 A. Except as provided in subsection C of this section, the
21 power of an agent or default surrogate commences when the individual
22 is found under subsection B of Section 4 of this act or by a court
23 to lack capacity to make a health care decision. The power shall
24 cease if the individual later is found to have capacity to make a

1 health care decision, or the individual objects under subsection C
2 of Section 5 of this act to the finding of lack of capacity under
3 subsection B of Section 4 of this act. Subject to subsection E of
4 Section 5 of this act, the power shall resume if:

5 1. The power ceased because the individual objected under
6 subsection C of Section 5 of this act; and

7 2. The finding of lack of capacity is confirmed under paragraph
8 4 of subsection D of Section 5 of this act, or a court finds that
9 the individual lacks capacity to make a health care decision.

10 B. An agent or default surrogate may request, receive, examine,
11 copy, and consent to the disclosure of medical and other health care
12 information about the individual if the individual would have the
13 right to request, receive, examine, copy, or consent to the
14 disclosure of the information.

15 C. A power of attorney for health care may provide that the
16 power of an agent under subsection B of this section commences on
17 appointment.

18 D. If no other person is authorized to do so, an agent or
19 default surrogate may apply for public or private health insurance
20 and benefits on behalf of the individual. An agent or default
21 surrogate who may apply for insurance and benefits does not, solely
22 by reason of the power, have a duty to apply for the insurance or
23 benefits.

24

1 E. An agent or default surrogate may not consent to voluntary
2 admission of the individual to a facility for mental health
3 treatment unless:

4 1. Voluntary admission is specifically authorized by the
5 individual in an advance health care directive in a record; and

6 2. The admission is for no more than the maximum of the number
7 of days specified in the directive.

8 F. Except as provided in subsection G of this section, an agent
9 or default surrogate may not consent to placement of the individual
10 in a nursing home, other than for short-term rehabilitation services
11 following hospitalization, if the individual objects to the
12 placement and a reasonable alternative living arrangement is
13 feasible.

14 G. If specifically authorized by the individual in an advance
15 health care directive in a record, an agent or default surrogate may
16 consent to placement of the individual in a nursing home even if a
17 reasonable alternative living arrangement is feasible.

18 SECTION 19. NEW LAW A new section of law to be codified
19 in the Oklahoma Statutes as Section 3100.19 of Title 63, unless
20 there is created a duplication in numbering, reads as follows:

21 A. If an individual has a long-term disability requiring
22 routine treatment by mechanical ventilation and a history of using
23 the treatment without objection, an agent or default surrogate may
24 not consent to withhold or withdraw the treatment unless:

1 1. The treatment is both not necessary to sustain the
2 individual's life and not necessary to maintain the individual's
3 health or well-being;

4 2. The individual has expressly authorized the withholding or
5 withdrawal in a health care instruction that has not been revoked;
6 or

7 3. The individual has experienced a life-threatening condition
8 that is not curable and is expected to cause the individual to die
9 within two weeks, even if treated, and the individual has not:

10 a. given a direction inconsistent with withholding or
11 withdrawal, or

12 b. communicated by verbal or nonverbal expression a
13 desire for mechanical ventilation.

14 B. An advance directive that is executed in this state shall
15 not be deemed to authorize the withholding or withdrawal of
16 artificially administered nutrition or hydration unless it
17 specifically authorizes the withholding or withdrawal of
18 artificially administered nutrition or hydration in the declarant's
19 own words or by a separate section, separate paragraph, or other
20 separate subdivision that deals only with nutrition or hydration and
21 which section, paragraph, or other subdivision is separately
22 initialed, separately signed, or otherwise separately marked by the
23 declarant.

1 C. Execution of an advance directive by an individual that
2 provides for the provision, withholding, or withdrawal of life-
3 sustaining treatment for the individual or for the appointment of
4 another to give directions to provide, withhold, or withdraw life-
5 sustaining treatment, executed in another state in compliance with
6 the laws of that state or of this state, shall be valid for purposes
7 of the this act to the extent the advance directive does not exceed
8 authorizations allowed under the laws of this state. No such
9 advance directive shall be deemed to authorize the withholding or
10 withdrawal of artificially administered nutrition or hydration
11 unless it specifically authorizes such withholding or withdrawal,
12 and the advance directive either:

13 1. Was executed by a person who was not a resident of this
14 state at the time of execution; or

15 2. Specifically authorizes the withholding or withdrawal of
16 artificially administered nutrition or hydration in the declarant's
17 own words or by a separate section, separate paragraph, or other
18 separate subdivision that deals only with nutrition or hydration,
19 and such section, paragraph, or other subdivision is separately
20 initialed, separately signed, or otherwise separately marked by the
21 person executing the advance directive.

22 D. Decisions by a default surrogate or agent to consent to
23 withholding or withdrawal of nutrition or hydration are subject to
24

1 the provisions of the Hydration and Nutrition for Incompetent
2 Patients Act.

3 E. A default surrogate may not make a health care decision if,
4 under other laws of this state, the decision:

5 1. May not be made by a guardian; or

6 2. May be made by a guardian only if the court appointing the
7 guardian specifically authorizes the guardian to make the decision.

8 SECTION 20. NEW LAW A new section of law to be codified
9 in the Oklahoma Statutes as Section 3100.20 of Title 63, unless
10 there is created a duplication in numbering, reads as follows:

11 A. An individual in a power of attorney for health care may
12 appoint multiple individuals as coagents. Unless the power of
13 attorney provides otherwise, each coagent may exercise independent
14 authority.

15 B. An individual in a power of attorney for health care may
16 appoint one or more individuals to act as alternate agents if a
17 predecessor agent resigns, dies, becomes disqualified, is not
18 reasonably available, or otherwise is unwilling or unable to act as
19 agent.

20 C. Unless the power of attorney provides otherwise, an
21 alternate agent has the same authority as the original agent:

22 1. At any time the original agent is not reasonably available
23 or is otherwise unwilling or unable to act, for the duration of the
24 unavailability, unwillingness, or inability to act; or

1 2. If the original agent and all other predecessor agents have
2 resigned or died or are disqualified from acting as agent.

3 SECTION 21. NEW LAW A new section of law to be codified
4 in the Oklahoma Statutes as Section 3100.21 of Title 63, unless
5 there is created a duplication in numbering, reads as follows:

6 No person shall be authorized as a default surrogate to make
7 health care decisions for a patient if that person:

8 1. Has been convicted of, pled guilty to, or pled no contest to
9 any violation of Section 843.1, 843.2, 843.4, or 843.5 of Title 21
10 of the Oklahoma Statutes;

11 2. Has been found to have committed abuse, verbal abuse, or
12 exploitation, as defined in Section 10-103 of Title 43A of the
13 Oklahoma Statutes, by a final State Department of Health or
14 Department of Human Services investigative finding, or by an
15 administrative law judge finding, unless that finding has been
16 overturned through judicial review; or

17 3. Has been criminally charged as described in subsection B of
18 Section 10-110.1 of Title 43A of the Oklahoma Statutes, unless the
19 person has been acquitted or those charges have been finally
20 dismissed.

21 SECTION 22. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.22 of Title 63, unless
23 there is created a duplication in numbering, reads as follows:

1 A. A responsible health care professional who is aware that an
2 individual has been found to lack capacity to make a decision shall
3 make a reasonable effort to determine if the individual has a
4 surrogate.

5 B. If possible before implementing a health care decision made
6 by a surrogate, a responsible health care professional, as soon as
7 reasonably feasible, shall communicate to the individual the
8 decision made and the identity of the surrogate.

9 C. A responsible health care professional who makes or is
10 informed of a finding that an individual lacks capacity to make a
11 health care decision or no longer lacks capacity, or that other
12 circumstances exist that affect a health care instruction or the
13 authority of a surrogate, as soon as reasonably feasible, shall:

14 1. Document the finding or circumstance in the individual's
15 medical record; and

16 2. If possible, communicate to the individual and the
17 individual's surrogate the finding or circumstance and that the
18 individual may object under subsection C of Section 5 of this act to
19 the finding under subsection B of Section 4 of this act.

20 D. A responsible health care professional who is informed that
21 an individual has created or revoked an advance health care
22 directive, or that a surrogate for an individual has been appointed,
23 designated, or disqualified, shall:

1 1. Document the information as soon as reasonably feasible in
2 the individual's medical record; and

3 2. If evidence of the directive, revocation, appointment,
4 designation, or disqualification is in a record, request a copy and,
5 upon receipt, cause the copy to be included in the individual's
6 medical record.

7 E. Except as provided in subsections F and G of this section, a
8 health care professional or health care institution providing health
9 care to an individual shall comply with:

10 1. A health care instruction given by the individual regarding
11 the individual's health care;

12 2. A reasonable interpretation by the individual's surrogate of
13 an instruction given by the individual; and

14 3. A health care decision for the individual made by the
15 individual's surrogate in accordance with Sections 17 and 18 of this
16 act to the same extent as if the decision had been made by the
17 individual at a time when the individual had capacity.

18 F. A health care professional or a health care institution may
19 refuse to provide health care consistent with a health care
20 instruction or health care decision if:

21 1. The instruction or decision is contrary to a policy of the
22 health care institution providing care to the individual that is
23 based expressly on reasons of conscience, and the policy was timely
24 communicated to the individual or to the individual's surrogate;

1 2. The care would require health care that is not available to
2 the professional or institution; or

3 3. Compliance with the instruction or decision would:

4 a. require the professional to provide care that is
5 contrary to the professional's religious belief or
6 moral conviction if other law permits the professional
7 to refuse to provide care for that reason,

8 b. require the professional or institution to provide
9 care that is contrary to generally accepted health
10 care standards applicable to the professional or
11 institution, or

12 c. violate a court order or other law.

13 G. A health care professional or health care institution that
14 refuses to provide care under subsection F of this section shall:

15 1. As soon as reasonably feasible, inform the individual, if
16 possible, and the individual's surrogate of the refusal;

17 2. Immediately make a reasonable effort to transfer the
18 individual to another health care professional or health care
19 institution that is willing to comply with the instruction or
20 decision; and

21 3. Either:

22 a. if care is refused under paragraph 1 or 2 of
23 subsection F of this section, provide life-sustaining
24 care and care needed to keep or make the individual

1 comfortable, consistent with accepted medical
2 standards to the extent feasible, until a transfer is
3 made, or

4 b. if care is refused under paragraph 3 of subsection F
5 of this section, provide life-sustaining care and care
6 needed to keep or make the individual comfortable,
7 consistent with accepted medical standards, until a
8 transfer is made.

9 H. As used in this section, "generally accepted health care
10 standards" and "accepted medical standards" shall not be interpreted
11 to supersede, or to authorize any violation of, the
12 Nondiscrimination in Treatment Act.

13 SECTION 23. NEW LAW A new section of law to be codified
14 in the Oklahoma Statutes as Section 3100.23 of Title 63, unless
15 there is created a duplication in numbering, reads as follows:

16 A. A guardian may refuse to comply with or revoke the
17 individual's advance health care directive only if the court
18 appointing the guardian expressly orders the noncompliance or
19 revocation.

20 B. Unless a court orders otherwise, a health care decision made
21 by an agent appointed by an individual subject to guardianship
22 prevails over a decision of the guardian appointed for the
23 individual.

1 SECTION 24. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.24 of Title 63, unless

3 there is created a duplication in numbering, reads as follows:

4 A. A health care professional or health care institution acting
5 in good faith shall not be subject to civil or criminal liability or
6 to discipline for unprofessional conduct for:

7 1. Complying with a health care decision made for an individual
8 by another person if compliance is based on a reasonable belief that
9 the person has authority to make the decision, including a decision
10 to withhold or withdraw health care;

11 2. Refusing to comply with a health care decision made for an
12 individual by another person if the refusal is based on a reasonable
13 belief that the person lacked authority or capacity to make the
14 decision;

15 3. Complying with an advance health care directive based on a
16 reasonable belief that the directive is valid;

17 4. Refusing to comply with an advance health care directive
18 based on a reasonable belief that the directive is not valid,
19 including a reasonable belief that the directive was not made by the
20 individual or, after its creation, was substantively altered by a
21 person other than the individual who created it;

22 5. Determining that an individual who otherwise might be
23 authorized to act as an agent or default surrogate is not reasonably
24 available; or

1 6. Complying with an individual's direction under subsection D
2 of Section 9 of this act.

3 B. An agent, default surrogate, or individual with a reasonable
4 belief that the individual is an agent or a default surrogate shall
5 not be subject to civil or criminal liability or to discipline for
6 unprofessional conduct for a health care decision made in a good
7 faith effort to comply with Section 17 of this act.

8 SECTION 25. NEW LAW A new section of law to be codified
9 in the Oklahoma Statutes as Section 3100.25 of Title 63, unless
10 there is created a duplication in numbering, reads as follows:

11 A. A person shall not:

12 1. Intentionally falsify, in whole or in part, an advance
13 health care directive;

14 2. For the purpose of frustrating the intent of the individual
15 who created an advance health care directive or with knowledge that
16 doing so is likely to frustrate the intent:

17 a. intentionally conceal, deface, obliterate, or delete
18 the directive or a revocation of the directive without
19 consent of the individual who created or revoked the
20 directive, or

21 b. intentionally withhold knowledge of the existence or
22 revocation of the directive from a responsible health
23 care professional or health care institution providing
24

1 health care to the individual who created or revoked
2 the directive;

3 3. Coerce or fraudulently induce an individual to create,
4 revoke, or refrain from creating or revoking an advance health care
5 directive or a part of a directive; or

6 4. Require or prohibit the creation or revocation of an advance
7 health care directive as a condition for providing health care.

8 B. An individual who is the subject of conduct prohibited under
9 subsection A of this section, or the individual's estate, shall have
10 a cause of action against a person who violates subsection A of this
11 section for statutory damages of Twenty-five Thousand Dollars
12 (\$25,000.00) or actual damages resulting from the violation,
13 whichever is greater.

14 C. Subject to subsection D of this section, an individual who
15 makes a health care instruction, or the individual's estate, shall
16 have a cause of action against a health care professional or health
17 care institution that intentionally violates Section 22 of this act
18 for statutory damages of Fifty Thousand Dollars (\$50,000.00) or
19 actual damages resulting from the violation, whichever is greater.

20 D. A health care professional who is an emergency medical
21 responder shall not be liable under subsection C of this section for
22 a violation of subsection E of Section 22 of this act if:

23 1. The violation occurs in the course of providing care to an
24 individual experiencing a health condition for which the

1 professional reasonably believes the care was appropriate to avoid
2 imminent loss of life or serious harm to the individual;

3 2. The failure to comply is consistent with accepted standards
4 of the profession of the professional; and

5 3. The provision of care does not begin in a health care
6 institution in which the individual resides or was receiving care.

7 E. In an action under this section, a prevailing plaintiff may
8 recover reasonable attorney fees, court costs, and other reasonable
9 litigation expenses.

10 F. A cause of action or remedy under this section shall be in
11 addition to any cause of action or remedy under any other law.

12 SECTION 26. NEW LAW A new section of law to be codified
13 in the Oklahoma Statutes as Section 3100.26 of Title 63, unless
14 there is created a duplication in numbering, reads as follows:

15 A. A physical or electronic copy of an advance health care
16 directive, revocation of an advance health care directive, or
17 appointment, designation, or disqualification of a surrogate has the
18 same effect as the original.

19 B. An individual may create a certified physical copy of an
20 advance health care directive or revocation of an advance health
21 care directive that is in electronic form by affirming under penalty
22 of perjury that the physical copy is a complete and accurate copy of
23 the directive or revocation.

1 SECTION 27. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.27 of Title 63, unless

3 there is created a duplication in numbering, reads as follows:

4 A. On petition of an individual, the individual's surrogate, a
5 health care professional or health care institution providing health
6 care to the individual, or a person interested in the welfare of the
7 individual, the court may:

8 1. Enjoin implementation of a health care decision made by an
9 agent or default surrogate on behalf of the individual, on a finding
10 that the decision is inconsistent with Section 17 or 18 of this act;

11 2. Enjoin an agent from making a health care decision for the
12 individual, on a finding that the individual's appointment of the
13 agent has been revoked or the agent:

14 a. is disqualified under subsection B of Section 8 or
15 under Section 21 of this act,

16 b. is unwilling or unable to comply with Section 17 of
17 this act, or

18 c. poses a danger to the individual's well-being;

19 3. Enjoin another individual from acting as a default
20 surrogate, on a finding that the other individual acting as a
21 default surrogate did not comply with Section 12 of this act or the
22 other individual:

23 a. is disqualified under Section 14 or 21 of this act,
24

1 b. is unwilling or unable to comply with Section 17 of
2 this act, or

3 c. poses a danger to the first individual's well-being,
4 and authorize another member of the classes identified in subsection
5 B of Section 12 of this act who is able and willing to comply with
6 the provisions of this act to serve as the individual's default
7 surrogate in place of the enjoined surrogate; or

8 4. Order implementation of a health care decision made:

9 a. by and for the individual, or

10 b. by an agent or default surrogate who is acting in
11 compliance with the powers and duties of the agent or
12 default surrogate.

13 B. In this act, advocacy for the provision, withholding, or
14 withdrawal of health care or mental health care to or from an
15 individual is not itself evidence that an agent or default
16 surrogate, or a potential agent or default surrogate, poses a danger
17 to the individual's well-being.

18 C. A proceeding under this section shall be governed by
19 Sections 2-101 et seq. and 3-101 et seq. of Title 30 of the Oklahoma
20 Statutes.

21 SECTION 28. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.28 of Title 63, unless
23 there is created a duplication in numbering, reads as follows:

1 A. This act shall not authorize mercy killing, assisted
2 suicide, or euthanasia, and is subject to the provisions of the
3 Assisted Suicide Prevention Act.

4 B. This act shall not affect other laws of this state governing
5 treatment for mental illness of an individual involuntarily
6 committed to a mental health care institution under Section 5-410 et
7 seq. of Title 43A of the Oklahoma Statutes.

8 C. Death of an individual caused by withholding or withdrawing
9 health care in accordance with this act shall not constitute a
10 suicide or homicide or legally impair or invalidate a policy of
11 insurance or an annuity providing a death benefit, notwithstanding
12 any term of the policy or annuity.

13 D. This act shall not create a presumption concerning the
14 intention of an individual who has not created an advance health
15 care directive, except as provided in the Hydration and Nutrition
16 for Incompetent Patients Act.

17 E. An advance health care directive created before, on, or
18 after the effective date of this act shall be interpreted in
19 accordance with the laws of this state, excluding the state's choice
20 of law rules, at the time the directive is implemented.

21 SECTION 29. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.29 of Title 63, unless
23 there is created a duplication in numbering, reads as follows:
24

1 In applying and construing this uniform act, a court shall
2 consider the promotion of uniformity of the law among jurisdictions
3 that enact it, except with regard to provisions of this act that
4 substantively differ from provisions in the act as proposed by the
5 Uniform Law Commission.

6 SECTION 30. NEW LAW A new section of law to be codified
7 in the Oklahoma Statutes as Section 3100.30 of Title 63, unless
8 there is created a duplication in numbering, reads as follows:

9 A. Subject to Section 19 of this act, an advance health care
10 directive created before the effective date of this act shall be
11 valid if it complies with this act or complied at the time of
12 creation with the laws of the state in which it was created.

13 B. Except as provided in subsection E of Section 28 of this
14 act, this act shall not affect the validity or effect of an act done
15 before the effective date of this act.

16 C. An individual who assumed authority to act as default
17 surrogate before the effective date of this act may continue to act
18 as default surrogate until the individual for whom the default
19 surrogate is acting has capacity or the default surrogate is
20 disqualified, whichever occurs first.

21 SECTION 31. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.31 of Title 63, unless
23 there is created a duplication in numbering, reads as follows:

1 This act applies to an advance health care directive created
2 before, on, or after the effective date of this act.

3 SECTION 32. NEW LAW A new section of law to be codified
4 in the Oklahoma Statutes as Section 3100.32 of Title 63, unless
5 there is created a duplication in numbering, reads as follows:

6 Upon motion by any party to enjoin the implementation of a
7 health care decision by a surrogate to provide, withhold, or
8 withdraw life sustaining care, the court shall issue an order
9 requiring that, pending a final decision on the merits and the
10 resolution of any appeal, the individual be provided with health
11 care which, in reasonable medical judgment, is necessary to prevent
12 the individual's death. The court shall not order the provision of
13 such care if doing so would require the denial of the same health
14 care to another individual.

15 SECTION 33. REPEALER 63 O.S. 2021, Sections 3101.1,
16 3101.2, 3101.3, 3101.4, 3101.5, 3101.6, 3101.7, 3101.8, 3101.9,
17 3101.10, 3101.11, as amended by Section 737, Chapter 486, O.S.L.
18 2025, 3101.12, 3101.13, 3101.14, 3101.15, 3101.16, 3102.4, as
19 amended by Section 15, Chapter 136, O.S.L. 2022, and 3102.5 (63 O.S.
20 Supp. 2025, Sections 3101.11 and 3102.4), are hereby repealed.

21 SECTION 34. REPEALER Sections 1, 2, 3, as amended by
22 Section 1, Chapter 154, O.S.L. 2023, 4, 5, as amended by Section 2,
23 Chapter 154, O.S.L. 2023, 6, 7, 8, 9, 10, 11, 12, and 13, Chapter
24 136, O.S.L. 2022 (63 O.S. Supp. 2025, Sections 3111.1, 3111.2,

1 3111.3, 3111.4, 3111.5, 3111.6, 3111.7, 3111.8, 3111.9, 3111.10,
2 3111.11, 3111.12, and 3111.13), are hereby repealed.

3 SECTION 35. This act shall become effective July 1, 2027."

4 Passed the Senate the 5th day of May, 2026.

5

6

Presiding Officer of the Senate

7

8 Passed the House of Representatives the ____ day of _____,
9 2026.

10

11

Presiding Officer of the House
of Representatives

12

13

14

15

16

17

18

19

20

21

22

23

24

1 ENGROSSED HOUSE
2 BILL NO. 1687

By: Roe of the House

3 and

4 Rosino of the Senate
5
6

7 An Act relating to advance health care directives;
8 creating the Uniform Health Care Decisions Act of
9 2025; providing definitions; specifying capacity;
10 providing for presumption of capacity; providing for
11 overcoming presumption; providing notice; authorizing
12 the right to object; providing for judicial review;
13 providing for health care instruction; establishing
14 power of attorney for health care; establishing
15 advance mental health care directive; providing for
16 certain conflicting health care directives; providing
17 an optional form; providing for a default surrogate;
18 providing for disagreement; providing for
19 disqualification; providing for revocation; providing
20 for validity; providing for conflict of law;
21 providing for duties; providing for powers; limiting
22 powers; providing for coagents and alternate agents;
23 providing for duties of health care professionals;
24 providing for responsibilities of health care
professionals and health care institutions; providing
for decisions by guardians; providing for immunity;
prohibiting conduct; providing for damages; providing
for judicial relief; providing for construction;
providing for application; providing for
codification; and providing an effective date.

21 BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

22 SECTION 36. NEW LAW A new section of law to be codified
23 in the Oklahoma Statutes as Section 3100 of Title 63, unless there
24 is created a duplication in numbering, reads as follows:

1 This act shall be known and may be cited as the "Uniform Health
2 Care Decisions Act of 2025".

3 SECTION 37. NEW LAW A new section of law to be codified
4 in the Oklahoma Statutes as Section 3100.1 of Title 63, unless there
5 is created a duplication in numbering, reads as follows:

6 As used in this act:

7 1. "Advance health care directive" means a power of attorney
8 for health care, health care instruction, or both. The term
9 includes an advance mental health care directive;

10 2. "Advance mental health care directive" means a power of
11 attorney for health care, health care instruction, or both, created
12 under Section 9 of this act;

13 3. "Agent" means an individual appointed under a power of
14 attorney for health care to make a health care decision for the
15 individual who made the appointment. The term includes a coagent or
16 alternate agent appointed under Section 20 of this act;

17 4. "Capacity" means having capacity under Section 3 of this
18 act;

19 5. "Cohabitant" means each of two individuals who have been
20 living together as a couple for at least one (1) year after each
21 became an adult or was emancipated and who are not married to one
22 another;

23

24

1 6. "Default surrogate" means an individual authorized under
2 Section 12 of this act to make a health care decision for another
3 individual;

4 7. "Electronic" means relating to technology having electrical,
5 digital, magnetic, wireless, optical, electromagnetic, or similar
6 capabilities;

7 8. "Family member" means a spouse, domestic partner, adult
8 child, parent, or grandparent, or an adult descendant of a spouse,
9 child, parent, or grandparent;

10 9. "Guardian" means a person appointed under other law by a
11 court to make decisions regarding the personal affairs of an
12 individual, which may include health care decisions. The term does
13 not include a guardian ad litem;

14 10. "Health care" means care or treatment or a service or
15 procedure to maintain, monitor, diagnose, or otherwise affect an
16 individual's physical or mental illness, injury, or condition. The
17 term includes mental health care;

18 11. "Health care decision" means a decision made by an
19 individual or the individual's surrogate regarding the individual's
20 health care, including:

- 21 a. selection or discharge of a health care professional
22 or health care institution,
23
24

1 b. approval or disapproval of a diagnostic test, surgical
2 procedure, medication, therapeutic intervention, or
3 other health care, and

4 c. direction to provide, withhold, or withdraw artificial
5 nutrition or hydration, mechanical ventilation, or
6 other health care;

7 12. "Health care institution" means a facility or agency
8 licensed, certified, or otherwise authorized or permitted by other
9 law to provide health care in this state in the ordinary course of
10 business;

11 13. "Health care instruction" means a direction, whether or not
12 in a record, made by an individual that indicates the individual's
13 goals, preferences, or wishes concerning the provision, withholding,
14 or withdrawal of health care. The term includes a direction
15 intended to be effective if a specified condition arises;

16 14. "Health care professional" means a physician or other
17 individual licensed, certified, or otherwise authorized or permitted
18 by other law of this state to provide health care in this state in
19 the ordinary course of business or the practice of the physician's
20 or individual's profession;

21 15. "Individual" means an adult or emancipated minor;

22 16. "Mental health care" means care or treatment or a service
23 or procedure to maintain, monitor, diagnose, or otherwise affect an
24

1 individual's mental illness or other psychiatric, psychological, or
2 psychosocial condition;

3 17. "Nursing home" means a nursing facility as defined in
4 Section 1919(a)(1) of the Social Security Act, 42 U.S.C., Section
5 1396r(a)(1), as amended, or skilled nursing facility as defined in
6 Section 1819(a)(1) of the Social Security Act, 42 U.S.C., Section
7 1395i-3(a)(1), as amended;

8 18. "Person" means an individual, estate, business or nonprofit
9 entity, government or governmental subdivision, agency, or
10 instrumentality, or other legal entity;

11 19. "Person interested in the welfare of the individual" means:

- 12 a. the individual's surrogate,
- 13 b. a family member of the individual,
- 14 c. the cohabitant of the individual,
- 15 d. a public entity providing health care case management
16 or protective services to the individual,
- 17 e. a person appointed under other law to make decisions
18 for the individual under a power of attorney for
19 finances, or
- 20 f. a person that has an ongoing personal or professional
21 relationship with the individual, including a person
22 that has provided educational or health care services
23 or supported decision making to the individual;

1 20. "Physician" means an individual authorized to practice
2 medicine from the State Board of Medical Licensure and Supervision
3 or the State Board of Osteopathic Examiners;

4 21. "Power of attorney for health care" means a record in which
5 an individual appoints an agent to make health care decisions for
6 the individual;

7 22. "Reasonably available" means being able to be contacted
8 without undue effort and being willing and able to act in a timely
9 manner considering the urgency of an individual's health care
10 situation. When used to refer to an agent or default surrogate, the
11 term includes being willing and able to comply with the duties under
12 Section 17 of this act in a timely manner considering the urgency of
13 an individual's health care situation;

14 23. "Record" means information:

- 15 a. inscribed on a tangible medium, or
- 16 b. stored in an electronic or other medium and
- 17 retrievable in perceivable form;

18 24. "Responsible health care professional" means:

- 19 a. a health care professional designated by an individual
- 20 or the individual's surrogate to have primary
- 21 responsibility for the individual's health care or for
- 22 overseeing a course of treatment, or
- 23 b. in the absence of a designation under subparagraph a
- 24 of this paragraph, or if the professional designated

1 under subparagraph a of this paragraph is not
2 reasonably available, a health care professional who
3 has primary responsibility for overseeing the
4 individual's health care or for overseeing a course of
5 treatment;

6 25. "Sign" means with present intent to authenticate or adopt a
7 record:

- 8 a. execute or adopt a tangible symbol, or
- 9 b. attach to or logically associate with the record an
10 electronic symbol, sound, or process;

11 26. "State" means a state of the United States, the District of
12 Columbia, Puerto Rico, the United States Virgin Islands, or any
13 other territory, or possession subject to the jurisdiction of the
14 United States. The term includes a federally recognized Indian
15 tribe;

16 27. "Supported decision making" means assistance, from one or
17 more persons of an individual's choosing, that helps the individual
18 make or communicate a decision, including by helping the individual
19 understand the nature and consequences of the decision; and

20 28. "Surrogate" means:

- 21 a. an agent,
 - 22 b. a default surrogate, or
 - 23 c. a guardian authorized to make health care decisions.
- 24

1 SECTION 38. NEW LAW A new section of law to be codified
2 in the Oklahoma Statutes as Section 3100.2 of Title 63, unless there
3 is created a duplication in numbering, reads as follows:

4 A. An individual has capacity for the purpose of this act if
5 the individual:

6 1. Is willing and able to communicate a decision independently
7 or with appropriate services, technological assistance, supported
8 decision making, or other reasonable accommodation; and

9 2. In making or revoking:

10 a. a health care decision, understands the nature and
11 consequences of the decision, including the primary
12 risks and benefits of the decision,

13 b. a health care instruction, understands the nature and
14 consequences of the instruction, including the primary
15 risks and benefits of the choices expressed in the
16 instruction, and

17 c. an appointment of an agent under a health care power
18 of attorney or identification of a default surrogate
19 under paragraph 1 of subsection B of Section 12 of
20 this act, recognizes the identity of the individual
21 being appointed or identified and understands the
22 general nature of the relationship of the individual
23 making the appointment or identification with the
24 individual being appointed or identified.

1 B. The right of an individual who has capacity to make a
2 decision about the individual's health care is not affected by
3 whether the individual creates or revokes an advance health care
4 directive.

5 SECTION 39. NEW LAW A new section of law to be codified
6 in the Oklahoma Statutes as Section 3100.3 of Title 63, unless there
7 is created a duplication in numbering, reads as follows:

8 A. An individual is presumed to have capacity to make or revoke
9 a health care decision, health care instruction, and power of
10 attorney for health care unless:

11 1. A court has found the individual lacks capacity to do so; or

12 2. The presumption is rebutted under subsection B of this
13 section.

14 B. Subject to Sections 5 and 6 of this act, a presumption under
15 subsection A of this section may be rebutted by a finding that the
16 individual lacks capacity:

17 1. Subject to subsection C of this section, made on the basis
18 of a contemporaneous examination by any of the following:

19 a. a physician,

20 b. a psychologist licensed or otherwise authorized to
21 practice in this state, or

22 c. an individual with training and expertise in the
23 finding of lack of capacity who is licensed or

24 otherwise authorized to practice in this state as:

- (1) a physician assistant,
 - (2) an advanced practice registered nurse, or
 - (3) a social worker, or
- d. a responsible health care professional not described in subparagraph a, b, or c of this paragraph if:
- (1) the individual about whom the finding is to be made is experiencing a health condition requiring a decision regarding health care treatment to be made promptly to avoid loss of life or serious harm to the health of the individual, and
 - (2) an individual listed in subparagraph a, b, or c of this paragraph is not reasonably available;

2. Made in accordance with accepted standards of the profession and the scope of practice of the individual making the finding and to a reasonable degree of certainty; and

3. Documented in a record signed by the individual making the finding that includes an opinion of the cause, nature, extent, and probable duration of the lack of capacity.

C. The finding under subsection B of this section may not be made by:

1. A family member of the individual presumed to have capacity;
2. The cohabitant of the individual or a descendant of the cohabitant; or

1 3. The individual's surrogate, a family member of the
2 surrogate, or a descendant of the surrogate.

3 D. If the finding under subsection B of this section was based
4 on a condition the individual no longer has or a responsible health
5 care professional subsequently has good cause to believe the
6 individual has capacity, the individual is presumed to have capacity
7 unless a court finds the individual lacks capacity or the
8 presumption is rebutted under subsection B of this section.

9 SECTION 40. NEW LAW A new section of law to be codified
10 in the Oklahoma Statutes as Section 3100.4 of Title 63, unless there
11 is created a duplication in numbering, reads as follows:

12 A. As soon as reasonably feasible, an individual who makes a
13 finding under subsection B of Section 4 of this act shall inform the
14 individual about whom the finding was made or the individual's
15 responsible health care professional of the finding.

16 B. As soon as reasonably feasible, a responsible health care
17 professional who is informed of a finding under subsection B of
18 Section 4 of this act shall inform the individual about whom the
19 finding was made and the individual's surrogate.

20 C. An individual found under subsection B of Section 4 of this
21 act to lack capacity may object to the finding:

22 1. By orally informing a responsible health care professional;
23
24

1 2. In a record provided to a responsible health care
2 professional or the health care institution in which the individual
3 resides or is receiving care; or

4 3. By another act that clearly indicates the individual's
5 objection.

6 D. If the individual objects under subsection C of this
7 section, the finding under subsection B of Section 4 of this act is
8 not sufficient to rebut a presumption of capacity under subsection A
9 of Section 4 of this act and the individual must be treated as
10 having capacity unless:

11 1. The individual withdraws the objection;

12 2. A court finds the individual lacks the presumed capacity;

13 3. The individual is experiencing a health condition requiring
14 a decision regarding health care treatment to be made promptly to
15 avoid imminent loss of life or serious harm to the health of the
16 individual; or

17 4. Subject to subsection B of this section, the finding is
18 confirmed by a second finding made by an individual authorized under
19 paragraph 1 of subsection B of Section 4 of this act who:

20 a. did not make the first finding,

21 b. is not a family member of the individual who made the
22 first finding, and

23 c. is not the cohabitant of the individual who made the
24 first finding or a descendant of the cohabitant.

1 E. A second finding that the individual lacks capacity under
2 paragraph 4 of subsection D of this section is not sufficient to
3 rebut the presumption of capacity if the individual is requesting
4 the provision or continuation of life-sustaining treatment and the
5 finding is being used to make a decision to withhold or withdraw the
6 treatment.

7 F. As soon as reasonably feasible, a health care professional
8 who is informed of an objection under subsection C of this section
9 shall:

10 1. Communicate the objection to a responsible health care
11 professional; and

12 2. Document the objection and the date of the objection in the
13 individual's medical record or communicate the objection and the
14 date of the objection to an administrator with responsibility for
15 medical records of the health care institution providing health care
16 to the individual, who shall document the objection and the date of
17 the objection in the individual's medical record.

18 SECTION 41. NEW LAW A new section of law to be codified
19 in the Oklahoma Statutes as Section 3100.5 of Title 63, unless there
20 is created a duplication in numbering, reads as follows:

21 A. An individual found under subsection B of Section 4 of this
22 act to lack capacity, a responsible health care professional, the
23 health care institution providing health care to the individual, or
24 a person interested in the welfare of the individual may petition

1 the court in the respective county in which the individual resides
2 or is located to determine whether the individual lacks capacity.

3 B. The court in which a petition under subsection A of this
4 section is filed shall appoint legal counsel to represent the
5 individual if the individual does not have legal counsel and a
6 guardian ad litem. The court shall hear the petition as soon as
7 possible, but not later than seven (7) days after the petition is
8 filed. As soon as possible, but not later than seven (7) days after
9 the hearing, the court shall determine whether the individual lacks
10 capacity. The court may determine the individual lacks capacity
11 only if the court finds by clear and convincing evidence that the
12 individual lacks capacity.

13 SECTION 42. NEW LAW A new section of law to be codified
14 in the Oklahoma Statutes as Section 3100.6 of Title 63, unless there
15 is created a duplication in numbering, reads as follows:

16 A. An individual may create a health care instruction that
17 expresses the individual's preferences for future health care,
18 including preferences regarding:

- 19 1. Health care professionals or health care institutions;
- 20 2. How a health care decision will be made and communicated;
- 21 3. Persons that should or should not be consulted regarding a
22 health care decision;
- 23 4. A person to serve as guardian for the individual if one is
24 appointed; and

1 5. An individual to serve as a default surrogate.

2 B. A health care professional to whom an individual
3 communicates or provides an instruction under subsection A of this
4 section shall document the instruction and the date of the
5 instruction in the individual's medical record or communicate the
6 instruction and date of the instruction to an administrator with
7 responsibility for medical records of the health care institution
8 providing health care to the individual, who shall document the
9 instruction and the date of the instruction in the individual's
10 medical record.

11 C. A health care instruction made by an individual that
12 conflicts with an earlier health care instruction made by the
13 individual, including an instruction documented in a medical order,
14 revokes the earlier instruction to the extent of the conflict.

15 D. A health care instruction may be in the same record as a
16 power of attorney for health care.

17 SECTION 43. NEW LAW A new section of law to be codified
18 in the Oklahoma Statutes as Section 3100.7 of Title 63, unless there
19 is created a duplication in numbering, reads as follows:

20 A. An individual may create a power of attorney for health care
21 to appoint an agent to make health care decisions for the
22 individual.

23 B. An individual is disqualified from acting as agent for an
24 individual who lacks capacity to make health care decisions if:

1 1. A court finds that the potential agent poses a danger to the
2 individual's well-being, even if the court does not issue a
3 restraining order against the potential agent; or

4 2. The potential agent is an owner, operator, employee, or
5 contractor of a nursing home, or other residential care facility, in
6 which the individual resides or is receiving care, unless the owner,
7 operator, employee, or contractor is a family member of the
8 individual, the cohabitant of the individual, or a descendant of the
9 cohabitant.

10 C. A health care decision made by an agent is effective without
11 judicial approval.

12 D. A power of attorney for health care must be in a record,
13 signed by the individual creating the power, and signed by an adult
14 witness who:

15 1. Reasonably believes the act of the individual to create the
16 power of attorney is voluntary and knowing; and

17 2. Is not:

18 a. the agent appointed by the individual,

19 b. the agent's spouse, or cohabitant, or

20 c. if the individual resides or is receiving care in a
21 nursing home, or other residential care facility, the
22 owner, operator, employee, or contractor of the
23 nursing home, or other residential care facility; and
24

1 3. Is present when the individual signs the power of attorney
2 or when the individual represents that the power of attorney
3 reflects the individual's wishes.

4 E. A witness under subsection D of this section is considered
5 present if the witness and the individual are:

6 1. Physically present in the same location;

7 2. Using electronic means that allow for real time audio and
8 visual transmission and communication in real time to the same
9 extent as if the witness and the individual were physically present
10 in the same location; or

11 3. Able to speak to and hear each other in real time through
12 audio connection if:

13 a. the identity of the individual is personally known to
14 the witness, or

15 b. the witness is able to authenticate the identity of
16 the individual by receiving accurate answers from the
17 individual that enables the authentication.

18 F. A power of attorney for health care may include a health
19 care instruction.

20 SECTION 44. NEW LAW A new section of law to be codified
21 in the Oklahoma Statutes as Section 3100.8 of Title 63, unless there
22 is created a duplication in numbering, reads as follows:

23 A. An individual may create an advance health care directive
24 that addresses only mental health care for the individual. The

1 directive may include a health care instruction, a power of attorney
2 for health care, or both.

3 B. A health care instruction under this section may include the
4 individual's:

5 1. General philosophy and objectives regarding mental health
6 care; and

7 2. Specific goals, preferences, and wishes regarding the
8 provision, withholding, or withdrawal of a form of mental health
9 care, including:

10 a. preferences regarding professionals, programs, and
11 facilities,

12 b. admission to a mental health facility, including
13 duration of admission,

14 c. preferences regarding medications,

15 d. refusal to accept a specific type of mental health
16 care, including a medication, and

17 e. preferences regarding crisis intervention.

18 C. A power of attorney for health care under this section may
19 appoint an agent to make decisions only for mental health care.

20 D. An individual may direct in an advance mental health care
21 directive that, if the individual is experiencing a psychiatric or
22 psychological event specified in the directive, the individual may
23 not revoke the directive or a part of the directive.

1 E. If an advance mental health care directive includes a
2 direction under subsection D of this section, the advance mental
3 health care directive must be in a record that is separate from any
4 other advance health care directive created by the individual and
5 signed by the individual creating the advance mental health care
6 directive and at least two adult witnesses who:

7 1. Attest that to the best of their knowledge the individual:

8 a. understood the nature and consequences of the
9 direction, including its risks and benefits, and

10 b. made the direction voluntarily and without coercion or
11 undue influence;

12 2. Are not:

13 a. the agent appointed by the individual,

14 b. the agent's spouse, or cohabitant, and

15 c. if the individual resides in a nursing home, or other
16 residential care facility, the owner, operator,
17 employee, or contractor of the nursing home, or other
18 residential care facility; and

19 3. Are physically present in the same location as the
20 individual.

21 SECTION 45. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.9 of Title 63, unless there
23 is created a duplication in numbering, reads as follows:

1 A. If a direction in an advance mental health care directive of
2 an individual conflicts with a direction in another advance health
3 care directive of the individual, the later direction revokes the
4 earlier direction to the extent of the conflict.

5 B. An appointment of an agent to make decisions only for mental
6 health care for an individual does not revoke an earlier appointment
7 of an agent to make other health care decisions for the individual.
8 A later appointment revokes the authority of an agent under the
9 earlier appointment to make decisions about mental health care
10 unless otherwise specified in the power of attorney making the later
11 appointment.

12 C. An appointment of an agent to make health care decisions for
13 an individual other than decisions about mental health care made
14 after appointment of an agent authorized to make only mental health
15 care decisions does not revoke the appointment of the agent
16 authorized to make only mental health care decisions.

17 SECTION 46. NEW LAW A new section of law to be codified
18 in the Oklahoma Statutes as Section 3100.10 of Title 63, unless
19 there is created a duplication in numbering, reads as follows:

20 The following form may be used to create an advance health care
21 directive:

22 ADVANCE HEALTH CARE DIRECTIVE

23 HOW YOU CAN USE THIS FORM

You can use this form if you wish to name someone to make health care decisions for you in case you cannot make decisions for yourself. This is called giving the person a power of attorney for health care. This person is called your agent.

You can also use this form to state your wishes, preferences, and goals for health care, and to say if you want to be an organ donor after you die.

YOUR NAME AND DATE OF BIRTH

Name :

Date of birth:

PART A: NAMING AN AGENT

This part lets you name someone else to make health care decisions for you. You may leave any item blank.

1. NAMING AN AGENT

I want the following person to make health care decisions for me
if I cannot make decisions for myself:

Name :

Optional contact information (it is helpful to include information such as address, phone, and email):

2. NAMING AN ALTERNATE AGENT

I want the following person to make health care decisions for me if I cannot and my agent is not able or available to make them for me:

Name :

Optional contact information (it is helpful to include information such as address, phone, and email):

3. LIMITING YOUR AGENT'S AUTHORITY

I give my agent the power to make all health care decisions for me if I cannot make those decisions for myself, except the following:

(If you do not add a limitation here, your agent will be able to make all health care decisions that an agent is permitted to make under state law.)

PART B: HEALTH CARE INSTRUCTIONS

This part lets you state your priorities for health care and to state types of health care you do and do not want.

1. INSTRUCTIONS ABOUT LIFE-SUSTAINING TREATMENT

This section gives you the opportunity to say how you want your agent to act while making decisions for you. You may mark or initial each choice. You also may leave any choice blank.

Treatment. Medical treatment needed to keep me alive but not needed for comfort or any other purpose should (mark or initial all that apply):

(☐) Always be given to me. (If you mark or initial this choice, you should not mark or initial other choices in this "treatment" section.)

(☐) Not be given to me if I have a condition that is not curable and is expected to cause my death soon, even if treated.

1 (____) Not be given to me if I am unconscious and I am not
2 expected to be conscious again.

3 (____) Not be given to me if I have a medical condition from
4 which I am not expected to recover that prevents me from
5 communicating with people I care about, caring for myself, and
6 recognizing family and friends.

7 (____) Other (write what you want or do not want):

8 Food and liquids. If I can't swallow and staying alive requires
9 me to get food or liquids through a tube or other means for the rest
10 of my life, then food or liquids should (mark or initial all that
11 apply):

12 (____) Always be given to me. (If you mark or initial this
13 choice, you should not mark or initial other choices in this "food
14 and liquids" section.)

15 (____) Not be given to me if I have a condition that is not
16 curable and is expected to cause me to die soon, even if treated.

17 (____) Not be given to me if I am unconscious and am not
18 expected to be conscious again.

19 (____) Not be given to me if I have a medical condition from
20 which I am not expected to recover that prevents me from
21 communicating with people I care about, caring for myself, and
22 recognizing family and friends.

23 (____) Other (write what you want or do not want):
24

1 Pain relief. If I am in significant pain, care that will keep
2 me comfortable but is likely to shorten my life should (mark or
3 initial all that apply):

4 (____) Always be given to me. (If you mark or initial this
5 choice, you should not mark or initial other choices in this "pain
6 relief" section.)

7 (____) Never be given to me. (If you mark or initial this
8 choice, you should not mark or initial other choices in this "pain
9 relief" section.)

10 (____) Be given to me if I have a condition that is not curable
11 and is expected to cause me to die soon, even if treated.

12 (____) Be given to me if I am unconscious and am not expected
13 to be conscious again.

14 (____) Be given to me if I have a medical condition from which
15 I am not expected to recover that prevents me from communicating
16 with people I care about, caring for myself, and recognizing family
17 and friends.

18 (____) Other (write what you want or do not want):

19 2. MY PRIORITIES

20 You can use this section to indicate what is important to you,
21 and what is not important to you. This information can help your
22 agent make decisions for you if you cannot. It also helps others
23 understand your preferences.

1 You may mark or initial each choice. You also may leave any
2 choice blank.

3 Staying alive as long as possible even if I have substantial
4 physical limitations is:

5 (____) Very important

6 (____) Somewhat important

7 (____) Not important

8 Staying alive as long as possible even if I have substantial
9 mental limitations is:

10 (____) Very important

11 (____) Somewhat important

12 (____) Not important

13 Being free from significant pain is:

14 (____) Very important

15 (____) Somewhat important

16 (____) Not important

17 Being independent is:

18 (____) Very important

19 (____) Somewhat important

20 (____) Not important

21 Having my agent talk with my family before making decisions
22 about my care is:

23 (____) Very important

24 (____) Somewhat important

1 (____) Not important

2 Having my agent talk with my friends before making decisions
3 about my care is:

4 (____) Very important

5 (____) Somewhat important

6 (____) Not important

7 3. OTHER INSTRUCTIONS

8 You can write in this section more information about your goals,
9 values, and preferences for treatment, including care you want or do
10 not want. You can also use this section to name anyone who you do
11 not want to make decisions for you under any conditions.

12 PART C: OPTIONAL SPECIAL POWERS AND GUIDANCE

13 This part lets you give your agent additional powers, and to
14 provide more guidance about your wishes. You may mark or initial
15 each choice. You also may leave any choice blank.

16 1. OPTIONAL SPECIAL POWERS

17 My agent can do the following things ONLY if I have marked or
18 initialed them below:

19 (____) Admit me as a voluntary patient to a facility for mental
20 health treatment for up to ____ days (write in the number of days
21 you want like 7, 14, 30 or another number). (If I do not mark or
22 initial this choice, my agent MAY NOT admit me as a voluntary
23 patient to this type of facility.)

1 (____) Place me in a nursing home for more than 100 days even
2 if my needs can be met somewhere else, I am not terminally ill, and
3 I object. (If I do not mark or initial this choice, my agent MAY
4 NOT do this.)

5 2. ACCESS TO MY HEALTH INFORMATION

6 My agent may obtain, examine, and share information about my
7 health needs and health care if I am not able to make decisions for
8 myself. If I mark or initial below, my agent may also do that at
9 any time my agent thinks it will help me.

10 (____) I give my agent permission to obtain, examine, and share
11 information about my health needs and health care whenever my agent
12 thinks it will help me.

13 3. FLEXIBILITY FOR MY AGENT

14 Mark or initial below if you want to give your agent flexibility
15 in following instructions you provide in this form. If you do not,
16 your agent must follow the instructions even if your agent thinks
17 something else would be better for you.

18 (____) I give my agent permission to be flexible in applying
19 these instructions if my agent thinks it would be in my best
20 interest based on what my agent knows about me.

21 4. NOMINATION OF GUARDIAN

22 You can say who you would want as your guardian if you needed
23 one. A guardian is a person appointed by a court to make decisions
24

1 for someone who cannot make decisions. Filling this out does NOT
2 mean you want or need a guardian.

3 If a court appoints a guardian to make personal decisions for
4 me, I want the court to choose:

5 (____) My agent named in this form. If my agent cannot be a
6 guardian, I want the alternate agent named in this form.

7 (____) Other (write who you would want and their contact
8 information):

9 PART D: ORGAN DONATION

10 This part lets you donate your organs after you die. You may
11 leave any item blank.

12 1. DONATION

13 You may mark or initial only one choice.

14 (____) I donate my organs, tissues, and other body parts after
15 I die, even if it requires maintaining treatments that conflict with
16 other instructions I have put in this form, EXCEPT for those I list
17 below (list any body parts you do NOT want to donate):

18 (____) I do not want my organs, tissues, or body parts donated
19 to anybody for any reason. (If you mark or initial this choice, you
20 should skip the "PURPOSE OF DONATION" section.)

21 2. PURPOSE OF DONATION

22 You may mark or initial all that apply. (If you do not mark or
23 initial any of the purposes below, your donation can be used for all
24 of them.)

Organs, tissues, or body parts that I donate may be used for:

() Transplant

() Therapy

() Research

() Education

() All of the above

PART E: SIGNATURES

YOUR SIGNATURE

Sign your name:

Today's date:

City/Town/Village and State (optional):

SIGNATURE OF A WITNESS

You need a witness if you are using this form to name an agent.

The witness must be an adult and cannot be the person you are naming as agent or the agent's spouse, or someone the agent lives with as a couple. If you live or are receiving care in a nursing home, the witness cannot be an employee or contractor of the home or someone who owns or runs the home.

Name of Witness:

Signature of Witness:

(Only sign as a witness if you think the person signing above is doing it voluntarily.)

Date witness signed:

PART F: INFORMATION FOR AGENTS

1 1. If this form names you as an agent, you can make decisions
2 about health care for the person who named you when the person
3 cannot make their own.

4 2. If you make a decision for the person, follow any
5 instructions the person gave, including any in this form.

6 3. If you do not know what the person would want, make the
7 decision that you think is in the person's best interest. To figure
8 out what is in the person's best interest, consider the person's
9 values, preferences, and goals if you know them or can learn them.
10 Some of these preferences may be in this form. You should also
11 consider any behavior or communication from the person that
12 indicates what the person currently wants.

13 4. If this form names you as an agent, you can also get and
14 share the person's health information. Unless the person has said
15 so in this form, you can get or share this information only when the
16 person cannot make decisions about the person's health care.

17 SECTION 47. NEW LAW A new section of law to be codified
18 in the Oklahoma Statutes as Section 3100.11 of Title 63, unless
19 there is created a duplication in numbering, reads as follows:

20 A. A default surrogate may make a health care decision for an
21 individual who lacks capacity to make health care decisions and for
22 whom an agent, or guardian authorized to make health care decisions,
23 has not been appointed or is not reasonably available.

1 B. Unless the individual has an advance health care directive
2 that indicates otherwise, a member of the following classes, in
3 descending order of priority, who is reasonably available and not
4 disqualified under Section 14 of this act, may act as a default
5 surrogate for the individual:

6 1. An adult the individual has identified, other than in a
7 power of attorney for health care, to make a health care decision
8 for the individual if the individual cannot make the decision;

9 2. The individual's spouse, unless:

10 a. a petition for annulment, divorce, dissolution of
11 marriage, legal separation, or termination has been
12 filed and not dismissed or withdrawn,

13 b. a decree of annulment, divorce, dissolution of
14 marriage, legal separation, or termination has been
15 issued,

16 c. the individual and the spouse have agreed in a record
17 to a legal separation, or

18 d. the spouse has abandoned the individual for more than
19 one (1) year;

20 3. The individual's adult child or parent;

21 4. The individual's cohabitant;

22 5. The individual's adult sibling;

23 6. The individual's adult grandchild or grandparent;

1 7. An adult not listed in paragraphs 1 through 6 of this
2 subsection who has assisted the individual with supported decision
3 making routinely during the preceding six (6) months;

4 8. The individual's adult stepchild not listed in paragraphs 1
5 through 7 of this subsection whom the individual actively parented
6 during the stepchild's minor years and with whom the individual has
7 an ongoing relationship; or

8 9. An adult not listed in paragraphs 1 through 8 of this
9 subsection who has exhibited special care and concern for the
10 individual and is familiar with the individual's personal values.

11 C. A responsible health care professional may require an
12 individual who assumes authority to act as a default surrogate to
13 provide a declaration in a record under penalty of perjury stating
14 facts and circumstances reasonably sufficient to establish the
15 authority.

16 D. If a responsible health care professional reasonably
17 determines that an individual who assumed authority to act as a
18 default surrogate is not willing or able to comply with a duty under
19 Section 17 of this act or fails to comply with the duty in a timely
20 manner, the professional may recognize the individual next in
21 priority under subsection B of this section as the default
22 surrogate.

23 E. A health care decision made by a default surrogate is
24 effective without judicial approval.

1 SECTION 48. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.12 of Title 63, unless

3 there is created a duplication in numbering, reads as follows:

4 A. A default surrogate who assumes authority under Section 12
5 of this act shall inform a responsible health care professional if
6 two or more members of a class under subsection B of Section 12 of
7 this act have assumed authority to act as default surrogates and the
8 members do not agree on a health care decision.

9 B. A responsible health care professional shall comply with the
10 decision of a majority of the members of the class with highest
11 priority under subsection B of Section 12 of this act who have
12 communicated their views to the professional and the professional
13 reasonably believes are acting consistent with their duties under
14 Section 17 of this act.

15 C. If a responsible health care professional is informed that
16 the members of the class who have communicated their views to the
17 professional are evenly divided concerning the health care decision,
18 the professional shall make a reasonable effort to solicit the views
19 of members of the class who are reasonably available but have not
20 yet communicated their views to the professional. The professional,
21 after the solicitation, shall comply with the decision of a majority
22 of the members who have communicated their views to the professional
23 and the professional reasonably believes are acting consistent with
24 their duties under Section 17 of this act.

1 D. If the class remains evenly divided after the effort is made
2 under subsection C of this section, the health care decision must be
3 made as provided by other law of this state regarding the treatment
4 of an individual who is found to lack capacity.

5 SECTION 49. NEW LAW A new section of law to be codified
6 in the Oklahoma Statutes as Section 3100.13 of Title 63, unless
7 there is created a duplication in numbering, reads as follows:

8 A. An individual for whom a health care decision would be made
9 may disqualify another individual from acting as default surrogate
10 for the first individual. The disqualification must be in a record
11 signed by the first individual or communicated verbally or
12 nonverbally to the individual being disqualified, another
13 individual, or a responsible health care professional.
14 Disqualification under this subsection is effective even if made by
15 an individual who lacks capacity to make an advance directive if the
16 individual clearly communicates a desire that the individual being
17 disqualified cannot make health care decisions for the individual.

18 B. An individual is disqualified from acting as a default
19 surrogate for an individual who lacks capacity to make health care
20 decisions if:

21 1. A court finds that the potential default surrogate poses a
22 danger to the individual's well-being, even if the court does not
23 issue a restraining order against the potential surrogate;

1 2. The potential default surrogate is an owner, operator,
2 employee, or contractor of a nursing home, or other residential care
3 facility, in which the individual is residing or receiving care
4 unless the owner, operator, employee, or contractor is a family
5 member of the individual, the cohabitant of the individual, or a
6 descendant of the cohabitant; or

7 3. The potential default surrogate refuses to provide a timely
8 declaration under subsection C of Section 12 of this act.

9 SECTION 50. NEW LAW A new section of law to be codified
10 in the Oklahoma Statutes as Section 3100.14 of Title 63, unless
11 there is created a duplication in numbering, reads as follows:

12 A. An individual may revoke the appointment of an agent, the
13 designation of a default surrogate, or a health care instruction in
14 whole or in part, unless:

15 1. A court finds the individual lacks capacity to do so;

16 2. The individual is found under subsection B of Section 4 of
17 this act to lack capacity to do so, and if the individual objects to
18 the finding, the finding is confirmed under paragraph 4 of
19 subsection D of Section 5 of this act; or

20 3. The individual created an advance mental health care
21 directive that includes the provision under subsection D of Section
22 9 of this act and the individual is experiencing the psychiatric or
23 psychological event specified in the directive.

1 B. Revocation under subsection A of this section may be by any
2 act of the individual that clearly indicates that the individual
3 intends to revoke the appointment, designation, or instruction,
4 including an oral statement to a health care professional.

5 C. Except as provided in Section 10 of this act, an advance
6 health care directive of an individual that conflicts with another
7 advance health care directive of the individual revokes the earlier
8 directive to the extent of the conflict.

9 D. Unless otherwise provided in an individual's advance health
10 care directive appointing an agent, the appointment of a spouse of
11 an individual as agent for the individual is revoked if:

12 1. A petition for annulment, divorce, dissolution of marriage,
13 legal separation, or termination has been filed and not dismissed or
14 withdrawn;

15 2. A decree of annulment, divorce, dissolution of marriage,
16 legal separation, or termination has been issued;

17 3. The individual and the spouse have agreed in a record to a
18 legal separation; or

19 4. The spouse has abandoned the individual for more than one
20 (1) year.

21 SECTION 51. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.15 of Title 63, unless
23 there is created a duplication in numbering, reads as follows:

1 A. An advance health care directive created outside this state
2 is valid if it complies with:

3 1. The law of the state specified in the directive or, if a
4 state is not specified, the state in which the individual created
5 the directive; or

6 2. This act.

7 B. A person may assume without inquiry that an advance health
8 care directive is genuine, valid, and still in effect, and may
9 implement and rely on it, unless the person has good cause to
10 believe the directive is invalid or has been revoked.

11 C. An advance health care directive, revocation of a directive,
12 or a signature on a directive or revocation may not be denied legal
13 effect or enforceability solely because it is in electronic form.

14 D. Evidence relating to an advance health care directive,
15 revocation of a directive, or a signature on a directive or
16 revocation may not be excluded in a proceeding solely because the
17 evidence is in electronic form.

18 E. This act does not affect the validity of an electronic
19 record or signature that is valid under Section 15-101 et seq. of
20 Title 12A of the Oklahoma Statutes.

21 F. If this act conflicts with other law of this state relating
22 to the creation, execution, implementation, or revocation of an
23 advance health care directive, this act prevails.

1 SECTION 52. NEW LAW A new section of law to be codified

2 in the Oklahoma Statutes as Section 3100.16 of Title 63, unless
3 there is created a duplication in numbering, reads as follows:

4 A. An agent or default surrogate has a fiduciary duty to the
5 individual for whom the agent or default surrogate is acting when
6 exercising or purporting to exercise a power under Section 18 of
7 this act.

8 B. An agent or default surrogate shall make a health care
9 decision in accordance with the direction of the individual in an
10 advance health care directive and other goals, preferences, and
11 wishes of the individual to the extent known or reasonably
12 ascertainable by the agent or default surrogate.

13 C. If there is not a direction in an advance health care
14 directive and the goals, preferences, and wishes of the individual
15 regarding a health care decision are not known or reasonably
16 ascertainable by the agent or default surrogate, the agent or
17 default surrogate shall make the decision in accordance with the
18 agent's or default surrogate's determination of the individual's
19 best interest.

20 D. In determining the individual's best interest under
21 subsection C of this section, the agent or default surrogate shall:

22 1. Give primary consideration to the individual's
23 contemporaneous communications, including verbal and nonverbal
24 expressions;

1 2. Consider the individual's values to the extent known or
2 reasonably ascertainable by the agent or default surrogate; and

3 3. Consider the risks and benefits of the potential health care
4 decision.

5 E. As soon as reasonably feasible, an agent or default
6 surrogate who is informed of a revocation of an advance health care
7 directive or disqualification of the agent or default surrogate
8 shall communicate the revocation or disqualification to a
9 responsible health care professional.

10 SECTION 53. NEW LAW A new section of law to be codified
11 in the Oklahoma Statutes as Section 3100.17 of Title 63, unless
12 there is created a duplication in numbering, reads as follows:

13 A. Except as provided in subsection C of this section, the
14 power of an agent or default surrogate commences when the individual
15 is found under subsection B of Section 4 of this act or by a court
16 to lack capacity to make a health care decision. The power ceases
17 if the individual later is found to have capacity to make a health
18 care decision, or the individual objects under subsection C of
19 Section 5 of this act to the finding of lack of capacity under
20 subsection B of Section 4 of this act. The power resumes if:

21 1. The power ceased because the individual objected under
22 subsection C of Section 5 of this act; and
23
24

1 2. The finding of lack of capacity is confirmed under paragraph
2 4 of subsection D of Section 5 of this act or a court finds that the
3 individual lacks capacity to make a health care decision.

4 B. An agent or default surrogate may request, receive, examine,
5 copy, and consent to the disclosure of medical and other health care
6 information about the individual if the individual would have the
7 right to request, receive, examine, copy, or consent to the
8 disclosure of the information.

9 C. A power of attorney for health care may provide that the
10 power of an agent under subsection B of this section commences on
11 appointment.

12 D. If no other person is authorized to do so, an agent or
13 default surrogate may apply for public or private health insurance
14 and benefits on behalf of the individual. An agent or default
15 surrogate who may apply for insurance and benefits does not, solely
16 by reason of the power, have a duty to apply for the insurance or
17 benefits.

18 E. An agent or default surrogate may not consent to voluntary
19 admission of the individual to a facility for mental health
20 treatment unless:

21 1. Voluntary admission is specifically authorized by the
22 individual in an advance health care directive in a record; and

23 2. The admission is for no more than the maximum of the number
24 of days specified in the directive.

1 F. Except as provided in subsection G of this section, an agent
2 or default surrogate may not consent to placement of the individual
3 in a nursing home if the placement is intended to be for more than
4 one hundred (100) days if:

- 5 1. An alternative living arrangement is reasonably feasible;
- 6 2. The individual objects to the placement; or
- 7 3. The individual is not terminally ill.

8 G. If specifically authorized by the individual in an advance
9 health care directive in a record, an agent or default surrogate may
10 consent to placement of the individual in a nursing home for more
11 than one hundred (100) days even if:

- 12 1. An alternative living arrangement is reasonably feasible;
- 13 2. The individual objects to the placement; and
- 14 3. The individual is not terminally ill.

15 SECTION 54. NEW LAW A new section of law to be codified
16 in the Oklahoma Statutes as Section 3100.18 of Title 63, unless
17 there is created a duplication in numbering, reads as follows:

18 A. If an individual has a long-term disability requiring
19 routine treatment by artificial nutrition, hydration, or mechanical
20 ventilation and a history of using the treatment without objection,
21 an agent or default surrogate may not consent to withhold or
22 withdraw the treatment unless:

- 23 1. The treatment is not necessary to sustain the individual's
24 life or maintain the individual's well-being;

1 2. The individual has expressly authorized the withholding or
2 withdrawal in a health care instruction that has not been revoked;
3 or

4 3. The individual has experienced a major reduction in health
5 or functional ability from which the individual is not expected to
6 recover, even with other appropriate treatment, and the individual
7 has not:

8 a. given a direction inconsistent with withholding or
9 withdrawal, or

10 b. communicated by verbal or nonverbal expression a
11 desire for artificial nutrition, hydration, or
12 mechanical ventilation.

13 B. A default surrogate may not make a health care decision if,
14 under other law of this state, the decision:

15 1. May not be made by a guardian; or

16 2. May be made by a guardian only if the court appointing the
17 guardian specifically authorizes the guardian to make the decision.

18 SECTION 55. NEW LAW A new section of law to be codified
19 in the Oklahoma Statutes as Section 3100.19 of Title 63, unless
20 there is created a duplication in numbering, reads as follows:

21 A. An individual in a power of attorney for health care may
22 appoint multiple individuals as coagents. Unless the power of
23 attorney provides otherwise, each coagent may exercise independent
24 authority.

1 B. An individual in a power of attorney for health care may
2 appoint one or more individuals to act as alternate agents if a
3 predecessor agent resigns, dies, becomes disqualified, is not
4 reasonably available, or otherwise is unwilling or unable to act as
5 agent.

6 C. Unless the power of attorney provides otherwise, an
7 alternate agent has the same authority as the original agent:

8 1. At any time the original agent is not reasonably available
9 or is otherwise unwilling or unable to act, for the duration of the
10 unavailability, unwillingness, or inability to act; or

11 2. If the original agent and all other predecessor agents have
12 resigned or died or are disqualified from acting as agent.

13 SECTION 56. NEW LAW A new section of law to be codified
14 in the Oklahoma Statutes as Section 3100.20 of Title 63, unless
15 there is created a duplication in numbering, reads as follows:

16 No person shall be authorized under Section 3102.4 of this title
17 to make health care decisions for a patient if that person:

18 1. Has been convicted of, pled guilty to, or pled no contest to
19 any violation of Section 843.1, 843.2, 843.4 or 843.5 of Title 21 of
20 the Oklahoma Statutes;

21 2. Has been found to have committed abuse, verbal abuse or
22 exploitation, as these terms are defined in Section 10-103 of Title
23 43A of the Oklahoma Statutes, by a final State Department of Health
24 or Department of Human Services investigative finding or by an

1 administrative law judge finding, unless that finding has been
2 overturned through judicial review; or

3 3. Has been criminally charged as described in subsection B of
4 Section 10-110.1 of Title 43A of the Oklahoma Statutes, unless the
5 person has been acquitted or those charges have been finally
6 dismissed.

7 SECTION 57. NEW LAW A new section of law to be codified
8 in the Oklahoma Statutes as Section 3100.21 of Title 63, unless
9 there is created a duplication in numbering, reads as follows:

10 A. A responsible health care professional who is aware that an
11 individual has been found to lack capacity to make a decision shall
12 make a reasonable effort to determine if the individual has a
13 surrogate.

14 B. If possible before implementing a health care decision made
15 by a surrogate, a responsible health care professional, as soon as
16 reasonably feasible, shall communicate to the individual the
17 decision made and the identity of the surrogate.

18 C. A responsible health care professional who makes or is
19 informed of a finding that an individual lacks capacity to make a
20 health care decision or no longer lacks capacity, or that other
21 circumstances exist that affect a health care instruction or the
22 authority of a surrogate, as soon as reasonably feasible, shall:

23 1. Document the finding or circumstance in the individual's
24 medical record; and

1 2. If possible, communicate to the individual and the
2 individual's surrogate the finding or circumstance and that the
3 individual may object under subsection C of Section 5 of this act to
4 the finding under subsection B of Section 4 of this act.

5 D. A responsible health care professional who is informed that
6 an individual has created or revoked an advance health care
7 directive, or that a surrogate for an individual has been appointed,
8 designated, or disqualified, shall:

9 1. Document the information as soon as reasonably feasible in
10 the individual's medical record; and

11 2. If evidence of the directive, revocation, appointment,
12 designation, or disqualification is in a record, request a copy and,
13 on receipt, cause the copy to be included in the individual's
14 medical record.

15 E. Except as provided in subsections F and G of this section, a
16 health care professional or health care institution providing health
17 care to an individual shall comply with:

18 1. A health care instruction given by the individual regarding
19 the individual's health care;

20 2. A reasonable interpretation by the individual's surrogate of
21 an instruction given by the individual; and

22 3. A health care decision for the individual made by the
23 individual's surrogate in accordance with Sections 17 and 18 of this
24

1 act to the same extent as if the decision had been made by the
2 individual at a time when the individual had capacity.

3 F. A health care professional or a health care institution may
4 refuse to provide health care consistent with a health care
5 instruction or health care decision if:

6 1. The instruction or decision is contrary to a policy of the
7 health care institution providing care to the individual that is
8 based expressly on reasons of conscience and the policy was timely
9 communicated to the individual or to the individual's surrogate;

10 2. The care would require health care that is not available to
11 the professional or institution; or

12 3. Compliance with the instruction or decision would:

13 a. require the professional to provide care that is
14 contrary to the professional's religious belief or
15 moral conviction if other law permits the professional
16 to refuse to provide care for that reason,

17 b. require the professional or institution to provide
18 care that is contrary to generally accepted health
19 care standards applicable to the professional or
20 institution, or

21 c. violate a court order or other law.

22 G. A health care professional or health care institution that
23 refuses to provide care under subsection F of this section shall:
24

1 1. As soon as reasonably feasible, inform the individual, if
2 possible, and the individual's surrogate of the refusal;

3 2. Immediately make a reasonable effort to transfer the
4 individual to another health care professional or health care
5 institution that is willing to comply with the instruction or
6 decision; and

7 3. Either:

8 a. if care is refused under paragraph 1 or 2 of
9 subsection F of this section, provide life-sustaining
10 care and care needed to keep or make the individual
11 comfortable, consistent with accepted medical
12 standards to the extent feasible, until a transfer is
13 made, or

14 b. if care is refused under paragraph 3 of subsection F
15 of this section, provide life-sustaining care and care
16 needed to keep or make the individual comfortable,
17 consistent with accepted medical standards, until a
18 transfer is made or, if the professional or
19 institution reasonably believes that a transfer cannot
20 be made, for at least ten (10) days after the refusal.

21 SECTION 58. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.22 of Title 63, unless
23 there is created a duplication in numbering, reads as follows:
24

1 A. A guardian may refuse to comply with or revoke the
2 individual's advance health care directive only if the court
3 appointing the guardian expressly orders the noncompliance or
4 revocation.

5 B. Unless a court orders otherwise, a health care decision made
6 by an agent appointed by an individual subject to guardianship
7 prevails over a decision of the guardian appointed for the
8 individual.

9 SECTION 59. NEW LAW A new section of law to be codified
10 in the Oklahoma Statutes as Section 3100.23 of Title 63, unless
11 there is created a duplication in numbering, reads as follows:

12 A. A health care professional or health care institution acting
13 in good faith is not subject to civil or criminal liability or to
14 discipline for unprofessional conduct for:

15 1. Complying with a health care decision made for an individual
16 by another person if compliance is based on a reasonable belief that
17 the person has authority to make the decision, including a decision
18 to withhold or withdraw health care;

19 2. Refusing to comply with a health care decision made for an
20 individual by another person if the refusal is based on a reasonable
21 belief that the person lacked authority or capacity to make the
22 decision;

23 3. Complying with an advance health care directive based on a
24 reasonable belief that the directive is valid;

1 4. Refusing to comply with an advance health care directive
2 based on a reasonable belief that the directive is not valid,
3 including a reasonable belief that the directive was not made by the
4 individual or, after its creation, was substantively altered by a
5 person other than the individual who created it;

6 5. Determining that an individual who otherwise might be
7 authorized to act as an agent or default surrogate is not reasonably
8 available; or

9 6. Complying with an individual's direction under subsection D
10 of Section 9 of this act.

11 B. An agent, default surrogate, or individual with a reasonable
12 belief that the individual is an agent or a default surrogate is not
13 subject to civil or criminal liability or to discipline for
14 unprofessional conduct for a health care decision made in a good
15 faith effort to comply with Section 17 of this act.

16 SECTION 60. NEW LAW A new section of law to be codified
17 in the Oklahoma Statutes as Section 3100.24 of Title 63, unless
18 there is created a duplication in numbering, reads as follows:

19 A. A person may not:

20 1. Intentionally falsify, in whole or in part, an advance
21 health care directive;

22 2. For the purpose of frustrating the intent of the individual
23 who created an advance health care directive or with knowledge that
24 doing so is likely to frustrate the intent:

- a. intentionally conceal, deface, obliterate, or delete the directive or a revocation of the directive without consent of the individual who created or revoked the directive, or
- b. intentionally withhold knowledge of the existence or revocation of the directive from a responsible health care professional or health care institution providing health care to the individual who created or revoked the directive;

3. Coerce or fraudulently induce an individual to create, revoke, or refrain from creating or revoking an advance health care directive or a part of a directive; or

4. Require or prohibit the creation or revocation of an advance health care directive as a condition for providing health care.

B. An individual who is the subject of conduct prohibited under subsection A of this section, or the individual's estate, has a cause of action against a person that violates subsection A of this section for statutory damages of Twenty-five Thousand Dollars (\$25,000.00) or actual damages resulting from the violation, whichever is greater.

C. Subject to subsection D of this section, an individual who makes a health care instruction, or the individual's estate, has a cause of action against a health care professional or health care institution that intentionally violates Section 22 of this act for

1 statutory damages of Fifty Thousand Dollars (\$50,000.00) or actual
2 damages resulting from the violation, whichever is greater.

3 D. A health care professional who is an emergency medical
4 responder is not liable under subsection C of this section for a
5 violation of subsection E of Section 22 of this act if:

6 1. The violation occurs in the course of providing care to an
7 individual experiencing a health condition for which the
8 professional reasonably believes the care was appropriate to avoid
9 imminent loss of life or serious harm to the individual;

10 2. The failure to comply is consistent with accepted standards
11 of the profession of the professional; and

12 3. The provision of care does not begin in a health care
13 institution in which the individual resides or was receiving care.

14 E. In an action under this section, a prevailing plaintiff may
15 recover reasonable attorney fees, court costs, and other reasonable
16 litigation expenses.

17 F. A cause of action or remedy under this section is in
18 addition to any cause of action or remedy under other law.

19 SECTION 61. NEW LAW A new section of law to be codified
20 in the Oklahoma Statutes as Section 3100.25 of Title 63, unless
21 there is created a duplication in numbering, reads as follows:

22 A. A physical or electronic copy of an advance health care
23 directive, revocation of an advance health care directive, or
24

1 appointment, designation, or disqualification of a surrogate has the
2 same effect as the original.

3 B. An individual may create a certified physical copy of an
4 advance health care directive or revocation of an advance health
5 care directive that is in electronic form by affirming under penalty
6 of perjury that the physical copy is a complete and accurate copy of
7 the directive or revocation.

8 SECTION 62. NEW LAW A new section of law to be codified
9 in the Oklahoma Statutes as Section 3100.26 of Title 63, unless
10 there is created a duplication in numbering, reads as follows:

11 A. On petition of an individual, the individual's surrogate, a
12 health care professional or health care institution providing health
13 care to the individual, or a person interested in the welfare of the
14 individual, the court may:

15 1. Enjoin implementation of a health care decision made by an
16 agent or default surrogate on behalf of the individual, on a finding
17 that the decision is inconsistent with Section 17 or 18 of this act;

18 2. Enjoin an agent from making a health care decision for the
19 individual, on a finding that the individual's appointment of the
20 agent has been revoked or the agent:

21 a. is disqualified under subsection B of Section 8 of
22 this act,

23 b. is unwilling or unable to comply with Section 17 of
24 this act, or

1 c. poses a danger to the individual's well-being;

2 3. Enjoin another individual from acting as a default
3 surrogate, on a finding that the other individual acting as a
4 default surrogate did not comply with Section 12 of this act or the
5 other individual:

6 a. is disqualified under Section 14 of this act,

7 b. is unwilling or unable to comply with Section 17 of
8 this act, or

9 c. poses a danger to the first individual's well-being; or

10 4. Order implementation of a health care decision made:

11 a. by and for the individual, or

12 b. by an agent or default surrogate who is acting in
13 compliance with the powers and duties of the agent or
14 default surrogate.

15 B. In this act, advocacy for the withholding or withdrawal of
16 health care or mental health care from an individual is not itself
17 evidence that an agent or default surrogate, or a potential agent or
18 default surrogate, poses a danger to the individual's well-being.

19 C. A proceeding under this section is governed by Sections 2-
20 101 et seq. and 3-101 et seq. of Title 30 of the Oklahoma Statutes.

21 SECTION 63. NEW LAW A new section of law to be codified
22 in the Oklahoma Statutes as Section 3100.27 of Title 63, unless
23 there is created a duplication in numbering, reads as follows:

1 A. This act does not authorize mercy killing, assisted suicide,
2 or euthanasia.

3 B. This act does not affect other law of this state governing
4 treatment for mental illness of an individual involuntarily
5 committed to a mental health care institution under Section 5-410 et
6 seq. of Title 43A of the Oklahoma Statutes.

7 C. Death of an individual caused by withholding or withdrawing
8 health care in accordance with this act does not constitute a
9 suicide or homicide or legally impair or invalidate a policy of
10 insurance or an annuity providing a death benefit, notwithstanding
11 any term of the policy or annuity.

12 D. This act does not create a presumption concerning the
13 intention of an individual who has not created an advance health
14 care directive.

15 E. An advance health care directive created before, on, or
16 after the effective date of this act must be interpreted in
17 accordance with law of this state, excluding the state's choice of
18 law rules, at the time the directive is implemented.

19 SECTION 64. NEW LAW A new section of law to be codified
20 in the Oklahoma Statutes as Section 3100.28 of Title 63, unless
21 there is created a duplication in numbering, reads as follows:

22 In applying and construing this uniform act, a court shall
23 consider the promotion of uniformity of the law among jurisdictions
24 that enact it.

1 SECTION 65. NEW LAW A new section of law to be codified
2 in the Oklahoma Statutes as Section 3100.29 of Title 63, unless
3 there is created a duplication in numbering, reads as follows:

4 A. An advance health care directive created before the
5 effective date of this act is valid if it complies with this act or
6 complied at the time of creation with the law of the state in which
7 it was created.

8 B. This act does not affect the validity or effect of an act
9 done before the effective date of this act.

10 C. An individual who assumed authority to act as default
11 surrogate before the effective date of this act may continue to act
12 as default surrogate until the individual for whom the default
13 surrogate is acting has capacity or the default surrogate is
14 disqualified, whichever occurs first.

15 SECTION 66. NEW LAW A new section of law to be codified
16 in the Oklahoma Statutes as Section 3100.30 of Title 63, unless
17 there is created a duplication in numbering, reads as follows:

18 This act applies to an advance health care directive created
19 before, on, or after the effective date of this act.

20 SECTION 67. REPEALER 63 O.S. 2021, Sections 3101.1,
21 3101.2, 3101.3, 3101.4, 3101.5, 3101.6, 3101.7, 3101.8, 3101.9,
22 3101.10, 3101.11, 3101.12, 3101.13, 3101.14, 3101.15, and 3101.16,
23 are hereby repealed.

24 SECTION 68. This act shall become effective November 1, 2025.

1 Passed the House of Representatives the 26th day of March, 2025.

2
3 _____
4 Presiding Officer of the House
5 of Representatives

6 Passed the Senate the _____ day of _____, 2025.

7
8 _____
9 Presiding Officer of the Senate